

ONTARIO
SUPERIOR COURT OF JUSTICE

B E T W E E N:)
)
SALVATORE MADONIA, PAUL) S. MacDonald and E. Swan, for
THOMAS MADONIA, KATHERYN) the Plaintiffs
ANNE MADONIA, a minor under)
the age of eighteen years by her)
litigation Guardian, Paul Thomas)
Madonia, ALEXANDER GREGORY)
MADONIA, a minor under the age)
of eighteen years by his litigation)
Guardian, Paul Thomas Madonia,)
LAURA THERESA LEWIS,)
Personally and WALKER MICHAEL)
SALVATORE LEWIS, a minor)
under the age of eighteen years by)
his litigation Guardian, Laura)
Theresa Lewis)
)
Plaintiffs)
)
- and -)
)
)
BRETT J.D. STEVENS) E. Lederman and M. Royce, for
) the Defendant
)
)
Defendant)
)
)
) HEARD: April 14, 15, 17, 21, 22,

June 4, June 18, 2008

REASONS FOR JUDGMENT

Coats J.

1. Overview

[1] On October 2, 2005 Laura Madonia died. She was 78 years old. The Plaintiffs are her husband, her adult children and her grandchildren. The Defendant, Dr. Brett Stevens, was Laura Madonia's cardiologist from December of 2000 to her death. The central issue in this trial is whether Dr. Stevens' care of Laura Madonia fell below the standard of care and caused or materially contributed to her death.

2. Summary of Positions

A) Summary of Plaintiffs' position

[2] The Plaintiffs' position is that Dr. Stevens fell below the standard of care expected of a reasonable prudent cardiologist in the following manner:

- a) Dr. Stevens interpreted Laura Madonia's electrocardiogram of July 15, 2005 as demonstrating that she was in atrial fibrillation, when she was not. As a consequence, Dr. Stevens prescribed anti-coagulation therapy, the drug Coumadin, which was unnecessary and which led directly to her death; and

b) Dr. Stevens implemented the anti-coagulation therapy inadequately, as follows:

- i) He delayed in informing Laura Madonia;
- ii) He did not have Laura Madonia come into his office to discuss the potential risk/benefit analysis between anti-coagulation therapy and stroke;
- iii) He did not accurately assess the risk of bleeding from the anti-coagulation therapy versus the benefit of avoiding stroke by having Laura Madonia and her risk of bleeding assessed by a gastroenterologist;
- iv) He did not carefully review with Laura Madonia in his office the risks and benefits;
- v) Dr. Stevens did not specifically outline the specific schedule of blood testing in a detailed, consistent and coherent fashion; and
- vi) He did not inform Laura Madonia about her next blood test.

[3] The consequences were that Laura Madonia's INR (international normalized ratio) reached an unacceptably high level which led directly to a severe gastrointestinal bleed which resulted in her death.

B) Summary of Defendant's Position

[4] Dr. Stevens' position is that he met the standard of care expected of a reasonable prudent cardiologist in the following manner:

- a) It was reasonable for him to interpret Laura Madonia's July 15, 2005 electrocardiogram as suggesting that Laura Madonia was likely in atrial fibrillation;
- b) Given Dr. Stevens' interpretation of the July 15, 2005 electrocardiogram, it was reasonable for Dr. Stevens to prescribe Coumadin; and
- c) Dr. Stevens' communication to Laura Madonia of the risks/benefits of Coumadin therapy and the associated monitoring was reasonable in the circumstances. The delay of approximately six weeks had no consequence to Laura Madonia. It was not necessary for Laura Madonia to come into the office. The risk/benefit analysis was reasonably and carefully conducted in a telephone conversation between Laura Madonia and Dr. Stevens. It

was not reasonably required that Laura Madonia's risk of bleeding be assessed by a gastroenterologist. He did outline a specific schedule of blood testing in a reasonable manner and his office assistant did advise Laura Madonia of her next blood test.

C) Causation

[5] There are also two causation issues. On the first causation issue, the Plaintiffs' position is that Dr. Stevens misinterpreted the electrocardiogram and, as a result, prescribed Coumadin which led to Laura Madonia's death. Dr. Stevens' position is that the prescription of Coumadin did not cause or materially contribute to Laura Madonia's death because Laura Madonia and her husband, Salvatore Madonia, failed to advise subsequent treating physicians of the fact that Coumadin had been prescribed.

[6] With respect to the second causation issue, Dr. Stevens' position is that the administration of the drug Propofol to Laura Madonia when she underwent an endoscopy on September 26, 2005 constituted an intervening event which caused Laura Madonia's death and for which Dr. Stevens is not liable. The Plaintiffs' position on the Propofol issue is twofold. Firstly, the Plaintiffs argue that the administration of Propofol as *novus actus interveniens* was not pleaded as a defence. Secondly, the Plaintiffs' position is that there was no evidence that the administration of Propofol fell below the standard of care and, but for the

Coumadin, she would not have developed the bleeding which led to her being taken to the endoscopy lab where Propofol was used as a sedative.

3. Issues Defined

[7] The standard of practice issues to be determined are as follows:

(a) Whether it was reasonable for Dr. Stevens to interpret the electrocardiogram tracing of July 15, 2005 as suggesting that Laura Madonia was likely in atrial fibrillation;

(b) Whether it was reasonable for Dr. Stevens to prescribe Coumadin to Laura Madonia given his interpretation of the July 15, 2005 electrocardiogram. It was acknowledged by Dr. Stevens in his examination for discovery, read into evidence, that he would not have prescribed Coumadin for Laura Madonia had he not interpreted the electrocardiogram as suggesting that she was in atrial fibrillation; and

(c) Whether Dr. Stevens' communication of the risks, side effects and benefits of Coumadin therapy and communication of the associated monitoring to Laura Madonia were reasonable in the circumstances.

[8] The causation issues to be determined are as follows:

(a) Whether Dr. Stevens' prescription of Coumadin caused or materially contributed to Laura Madonia's death or if Laura Madonia's failure to advise subsequent treating physicians that she was on Coumadin caused or materially contributed to her death; and

(b) Whether the administration of the drug Propofol during an endoscopic procedure at Joseph Brant Hospital caused Laura Madonia's death and for which administration Dr. Stevens is not liable.

4. Brief Summary of Undisputed Facts

[9] Laura Madonia was born on May 8, 1927 and died October 2, 2005. She married Salvatore Madonia on June 11, 1950. They have two adult children, Paul Thomas Madonia and Laura Theresa Lewis. Laura Madonia and Salvatore Madonia have three grandchildren, Katheryn Anne Madonia, Alexander Gregory Madonia and Walker Michael Salvatore Lewis.

[10] A brief synopsis of Laura Madonia's pertinent medical history is as follows:

1. She suffered from hypertrophic cardiomyopathy. She had a thickening of the heart muscle in the septum area of the heart. This condition can obstruct blood flow because of a flipping forward of the mitral valve

leaflet. Laura Madonia was on medication to suppress the force of the excessively contractile muscle.

2. She had heart block. This is an electrical disturbance in the heart and, because of this, she had a pacemaker inserted around 1997.

3. She suffered from congestive heart failure. For this she took diuretics.

4. She had an enlarged left atrium.

5. She had a retinal embolus occlusion in 1999.

6. She may have had some hardening of the coronary arteries.

7. She had hypothyroidism.

8. She had cataract surgery in 2005.

9. In the mid 1990's she had suffered gastrointestinal bleeding and was diagnosed with "watermelon stomach". The treatment included the removal of part of her stomach. Watermelon stomach is very superficial, larger than normal, blood vessels on the surface of the stomach lining, that predispose a person to bleeding.

10. She had a little bit of calcification of the mitral apparatus and a small amount of leak at the mitral value, called mitral regurgitation.

[11] In 1999 and 2000 in relation to the retinal embolism, Laura Madonia was prescribed Coumadin therapy. This was while she lived in Atlanta. She reported to Dr. Stevens that she had significant bleeding from the G.I. tract and the Coumadin was stopped. She told Dr. Stevens that she had been told not to take blood thinners again. This information is recorded in Dr. Stevens' letter to Dr. Graham dated December 5, 2000.

[12] In 2000 when the Madonias moved back to Ontario, Dr. Deborah Graham became Laura Madonia's family physician. Dr. Graham referred Laura Madonia to Dr. Stevens who began as Laura Madonia's cardiologist in late 2000.

[13] The undisputed chronology of events from July 15, 2005 to the date of Laura Madonia's death is as follows:

1. On July 15, 2005 Laura Madonia and her husband attended at Dr. Stevens' office and Laura Madonia had an electrocardiogram. Dr. Stevens met with the Madonias after the electrocardiogram and Dr. Stevens did not tell Laura Madonia that she was in atrial fibrillation.
2. Laura Madonia had not previously been in atrial fibrillation.

3. Dr. Stevens wrote to Dr. Graham by letter dated July 16, 2005. He reported that the twelve lead electrocardiogram showed atrial fibrillation.

4. On August 31, 2005 Dr. Stevens wrote to Dr. Wigle requesting his assistance and opinion regarding the ongoing treatment of Laura Madonia. In this letter, Dr. Stevens wrote, "I do notice on reviewing things today that she was actually in atrial fibrillation on the last ECG. I do not recall having seen this issue with her previously".

5. On September 6, 2005 Dr. Stevens telephoned Laura Madonia. She was prescribed 2.5 milligrams of Coumadin once daily with INR testing in approximately 5 days and weekly according to Dr. Stevens' note of September 6, 2005 and with testing in three days according to the written prescription of the same date.

6. Dr. Stevens signed a prescription for Coumadin dated September 6, 2005 indicating INR testing in three days.

7. Laura Madonia went to MDS Laboratory Services on September 9, 2005 for INR testing. Her level was 1.1, lower than the therapeutic range.

8. On September 13, 2005 Joann Rasmussen, Dr. Stevens' assistant, telephoned the Madonia home and spoke with Mr. Madonia. The

Coumadin dose was changed to 2.5 milligrams one day, alternating with 5 milligrams the following day.

9. On September 16, 2005 Laura Madonia attended at Joseph Brant Hospital complaining of difficulty swallowing. She did not report being on Coumadin.

10. On September 26, 2005 Laura Madonia re-attended at Joseph Brant Hospital suffering from bleeding. She was bringing up blood and passing blood through her rectum. Her Coumadin level was 6.7, well above the therapeutic range. The therapeutic range is normally between 2 and 3. Her change in haemoglobin levels between May 2005 and September 26, 2005 suggest that she had lost 4 to 5 units of blood, between a litre and a litre and a half of blood. She was sent to endoscopy so that the source of the bleeding could be determined. The endoscopy revealed multiple small ulcerations in the stomach. During the procedure, she went into cardiac arrest. She was revived, but did not regain consciousness. She was kept on life support. On October 2, 2005, after consultation with the doctors, the family decided to withdraw treatment. Laura Madonia died.

[14] This is not a complete chronology as I have only summarized undisputed facts. My findings related to the disputed facts will be detailed in the analysis section of these Reasons.

5. The Law on Standard Care and Conflicting Expert Evidence

[15] The law applicable to the standard of practice issues in medical malpractice litigation was set out by the Ontario Court of Appeal in *Crits v. Sylvester*, [1956] O.R. 132, affirmed 1956 CarswellOnt 84 (S.C.C.), at page five, paragraphs 13 and 14 as follows:

Every medical practitioner must bring to his task a reasonable degree of skill and knowledge and must exercise a reasonable degree of care. He is bound to exercise that degree of care and skill which could reasonably be expected of a normal, prudent practitioner of the same experience and standing, and if he holds himself out as a specialist, a higher degree of skill is required of him than of one who does not profess to be so qualified by special training and ability.

I do not believe that the standard of care required of a medical practitioner has been more clearly or succinctly stated than by Lord Hewart C.J. in *Rex v. Bateman* (1925), 41 T.L.R. 557 at 559: "If a person holds himself out as possessing special skill and knowledge and he is consulted, as possessing such skill and knowledge, by or on behalf of a patient, he owes a duty to the patient to use due caution in undertaking the treatment. If he accepts the responsibility and undertakes the treatment and the patient submits to his direction and treatment accordingly, he owes a duty to the patient to use diligence, care, knowledge, skill and caution in administering the treatment ... The law requires a fair and reasonable standard of care and competence."

[16] The Supreme Court of Canada in *ter Neuzen v. Korn*, 1995 CarswellBC 593, beginning on page 12 defined the applicable standard as follows:

1. Standard of Care and Evidence of Standard Practice

46 It is well settled that physicians have a duty to conduct their practice in accordance with the conduct of a prudent and diligent doctor in the same circumstances. In the case of a specialist, such as a gynaecologist and obstetrician, the doctor's behaviour must be assessed in light of the conduct of other ordinary specialists, who possess a reasonable level of knowledge, competence and skill expected of professionals in Canada, in that field. A specialist, such as the respondent, who holds himself out as possessing a special degree of skill and knowledge, must exercise the degree of skill of an average specialist in his field: see *Wilson v. Swanson*, [1956] S.C.R. 804, at p. 817, *Lapointe v. Hôpital Le Gardeur*, [1992] 1 S.C.R. 351, at p. 361, and *McCormick v. Marcotte*, [1972] S.C.R. 18.

47 It is also particularly important to emphasize, in the context of this case, that the conduct of physicians must be judged in the light of the knowledge that ought to have been reasonably possessed at the time of the alleged act of negligence. As Denning L.J. eloquently stated in *Roe v. Ministry of Health*, [1954] 2 All E.R. 131 (C.A.), at p. 137, "[w]e must not look at the 1947 accident with 1954 spectacles". That is, courts must not, with the benefit of hindsight, judge too harshly doctors who act in accordance with prevailing standards of professional knowledge. This point was also emphasized by this Court in *Lapointe*, supra, at pp. 362-63:

... courts should be careful not to rely upon the perfect vision afforded by hindsight. In order to evaluate a particular exercise of judgment fairly, the doctor's limited ability to foresee future events when determining a course of conduct must be borne in mind. Otherwise, the doctor will not be assessed according to the norms of the average doctor of reasonable ability in the same circumstances, but rather will be held accountable for mistakes that are apparent only after the fact.

No issue is taken with this proposition which was applied both in the trial judge's charge to the jury and by the Court of Appeal.

51 It is generally accepted that when a doctor acts in accordance with a recognized and respectable practice of the profession, he or she will not be found to be negligent. This is because courts do not ordinarily have the expertise to tell professionals that they are not behaving appropriately in their field. In a sense, the medical profession as a whole is assumed to have adopted procedures which are in the best interests of patients and are not inherently negligent. As L'Heureux-Dubé J. stated in *Lapointe*, in the context of the Quebec Civil Code (at pp. 363-64):

Given the number of available methods of treatment from which medical professionals must at times choose, and the distinction between error and fault, a doctor will not be found liable if the diagnosis and treatment given to a patient correspond to those recognized by medical science at the time, even in the face of competing theories. As expressed more eloquently by André Nadeau in "*La responsabilité médicale*" (1946), 6 R. du B. 153, at p. 155:

[Translation] The courts do not have jurisdiction to settle scientific disputes or to choose among divergent opinions of physicians on certain subjects. They may only make a finding of fault where a violation of universally accepted rules of medicine has occurred. The courts should not involve themselves in controversial questions of assessment having to do with diagnosis or the treatment of preference. [Emphasis added.]

52 In *The Law of Torts* (7th ed. 1987), Professor Fleming observed the following with respect to the role of standard practice, at p. 109:

Conformity with general practice, on the other hand, usually dispels a charge of negligence. It tends to show what others in the same "business"

considered sufficient, that the defendant could not have learnt how to avoid the accident by the example of others, that most probably no other practical precautions could have been taken, and that the impact of an adverse judgment (especially in cases involving industry or a profession) will be industry-wide and thus assume the function of a "test case". Finally, it underlines the need for caution against passing too cavalierly upon the conduct and decision of experts.

All the same, even a common practice may itself be condemned as negligent if fraught with obvious risks. [Emphasis added.]

53 With respect to the medical profession in particular, Professor Fleming noted, at p. 110:

Common practice plays its most conspicuous role in medical negligence actions. Conscious at once of the layman's ignorance of medical science and apprehensive of the impact of jury bias on a peculiarly vulnerable profession, courts have resorted to the safeguard of insisting that negligence in diagnosis and treatment (including disclosure of risks) cannot ordinarily be established without the aid of expert testimony or in the teeth of conformity with accepted medical practice. However there is no categorical rule. Thus an accepted practice is open to censure by a jury (nor expert testimony required) at any rate in matters not involving diagnostic or clinical skills, on which an ordinary person may presume to pass judgment sensibly, like omission to inform the patient of risks, failure to remove a sponge, an explosion set-off by an admixture of ether vapour and oxygen or injury to a patient's body outside the area of treatment. [Emphasis added. Footnotes omitted.]

54 It is evident from the foregoing passage that while conformity with common practice will generally exonerate physicians of any complaint of negligence, there are certain situations where the standard practice itself may be found to be negligent. However, this will only be where the standard practice is "fraught with obvious risks" such that anyone is capable of finding it negligent, without the necessity of judging matters requiring diagnostic or clinical expertise.

55 In *Roberge v. Bolduc*, [1991] 1 S.C.R. 374, this Court had the opportunity to address this issue in the context of the civil responsibility of a notary under the Quebec Civil Code. In that case, it was recognized that where a custom of a profession ignores the elementary dictates of caution, it is open to a court to find the professional person negligent. Thus, even if a doctor practises in accordance with common professional practice, he will be liable if that practice is wanting. As L'Heureux-Dubé J. stated at pp. 436-37:

This brief overview of both doctrine and jurisprudence indicates that courts have discretion to assess liability despite uncontradicted evidence of common professional practice at the relevant time. The standard, in regard to the particular facts of each case, must still be that of a reasonable professional in such circumstances.

It may very well be that the professional practice reflects prudent and diligent conduct. One would hope that if a certain practice has developed amongst professionals in regard to a particular professional act, such practice is in accordance with a prudent course of action. The fact that a professional has followed the practice of his or her peers may be strong evidence of reasonable and diligent conduct, but it is not determinative. If the practice is not in accordance with the general standards of liability, i.e., that one must act in a reasonable manner, then the professional who adheres to such a practice can be found liable, depending on the facts of each case. [Emphasis in original.]

56 The foregoing principles were also endorsed by this Court in *Waldick v. Malcolm*, [1991] 2 S.C.R. 456. Thus, it is apparent that conformity with standard practice in a profession does not necessarily insulate a doctor from negligence where the standard practice itself is negligent. The question that remains is under what circumstances will a professional standard practice be judged negligent? It seems that it is only where the practice does not conform with basic care which is easily understood by the ordinary person who has no particular expertise in the practices of the profession. That is, as Professor Fleming suggests, where the common practice is fraught with danger, a judge or a jury may find that the practice is itself negligent.

57 As was observed in *Lapointe*, courts should not involve themselves in resolving scientific disputes which require the expertise of the profession. Courts and juries do not have the necessary expertise to assess technical matters relating to the diagnosis or treatment of patients. Where a common and accepted course of conduct is adopted based on the specialized and technical expertise of professionals, it is unsatisfactory for a finder of fact to conclude that such a standard was inherently negligent. On the other hand, matters falling within the ordinary common sense of juries can be judged to be negligent. For example, where there are obvious existing alternatives which any reasonable person would utilize in order to avoid a risk, one could conclude that the failure to adopt such measures is negligent notwithstanding that it is the prevailing practice among practitioners in that area.

64 I conclude from the foregoing that, as a general rule, where a procedure involves difficult or uncertain questions of medical treatment or complex, scientific or highly technical matters that are beyond the ordinary experience and understanding of a judge or jury, it will not be open to find a standard medical practice negligent. On the other hand, as an exception to the general rule, if a standard practice fails to adopt obvious and reasonable precautions which are readily apparent to the ordinary finder of fact, then it is no excuse for a practitioner to claim that he or she was merely conforming to such a negligent common practice.

[17] In this case, there are conflicting opinions of experts as to whether Dr. Stevens fell below the standard of care. Dr. Gerald Wisenberg testified on behalf of the Plaintiffs. Dr. Ronald Baigrie testified on behalf of Dr. Stevens.

[18] Justice Power addresses the issue of conflicting opinions of experts in *Crawford v. Penney* 2003 CarswellOnt 82 (ON.S.C.). Beginning at p. 59, paragraph 241, Justice Power quotes from the *Brain v. Mador*, (1985), 9 O.A.C. 87, leave to appeal refused (1985), 64 N.R. 240 (note), (S.C.C.), a decision of the Ontario Court of Appeal, as follows:

Failure to exercise the ordinary skill of a doctor (in the appropriate specialty, if he be a specialist), is necessary. The last sentence quoted is consistent with the test applied in Canada in medical malpractice cases where negligence is established by proof that the specialist failed to possess or to exercise the reasonable degree of learning and skill possessed by the average specialist in the field. See *Wilson v. Swanson*, [1956] S.C.R. 803, 817. Professional opinions expressed have an important bearing on the latter test but, in my view, it is for the trier of fact to weigh the conflicting testimony and ultimately to assess the weight to be given to the evidence.

[19] In the same decision at paragraph 242, reference is made to the following section in the Alberta Court of Appeal decision in *Kehler v. Myles* (1988), 92 A.R., 345 (Alberta Court of Appeal):

There is no necessitated dismissal of a medical negligence claim simply because honest and competent experts disagree over a doctor's diagnosis or treatment. Disagreement notwithstanding, the only question to be answered remains, has negligence been established under proof by a preponderance of evidence that the specialist failed to possess or to exercise a reasonable degree of learning or skill possessed by the average specialist in the field?

[20] Justice Power went on to say at page 61, paragraph 248, the following:

I conclude from the foregoing that, indeed, the appropriate standard of care is determined by the trier of fact. Where there are conflicting expert opinions, the trier of fact must weigh the conflicting testimony and ultimately assess the weight to be given to the evidence. "There is no necessitated dismissal of a medical negligence claim simply because honest and competent experts disagree over a doctor's diagnosis or treatment."

6. Analysis and Findings re: Standard of Care - Dr. Stevens' reading of the July 15, 2005 Electrocardiogram

[21] Both Dr. Wisenberg, the Plaintiffs' expert, and Dr. Baigrie, Dr. Stevens' expert, are experienced cardiologists. They both defined normal rhythm, sinus rhythm and paced rhythm. These terms describe the following:

1. Sinus rhythm is normal rhythm. The electrical activity impulse in the heart begins in the sinus node and descends through the atria, causing them to contract and prime the ventricles, and then proceeds through the atrioventricular node into the ventricles or pumping chambers of the heart, causing the left ventricle to squeeze and contract to expel blood into the rest of the body and the right ventricle to expel blood into the lungs.

2. Atrial fibrillation is an irregular heart rhythm. There is a loss of the sinus node function. The atrium receives electrical activity in a random chaotic manner with the result that the atrium quivers and does not squeeze down properly and therefore does not perform its

function of priming the ventricles with blood. The amount of blood the heart pumps out is diminished.

3. Paced rhythm occurs with a pacemaker in place. The pacemaker governs the heart rhythm in accordance with a preset program.

[22] Dr. Wisenberg's evidence on whether Dr. Stevens fell below the standard of care regarding the July 15, 2005 electrocardiogram can be summarized as follows:

1. The July 15, 2005 electrocardiogram did not indicate or demonstrate that Laura Madonia was in atrial fibrillation. It showed she was in sinus rhythm with the pacemaker firing into the ventricles (sinus rhythm with a properly functioning pacemaker).
2. The July 15, 2005 electrocardiogram did not present a confusing picture. It required time to look over it carefully, but after it was looked at carefully it is a relatively straightforward electrocardiogram.
3. Dr. Stevens, in interpreting that this electrocardiogram demonstrated that Laura Madonia was in atrial fibrillation, fell below the standard of care of a reasonably trained and prudent cardiologist. Dr. Wisenberg's opinion was that this electrocardiogram, with careful usual review applied by a

cardiologist, should be interpreted as not demonstrating that Laura Madonia was in atrial fibrillation.

[23] Dr. Baigrie's evidence on whether Dr. Stevens fell below the standard of care regarding the July 15, 2005 electrocardiogram can be summarized as follows:

1. The July 15, 2005 electrocardiogram was a very difficult tracing.
2. From reviewing the electrocardiogram, a cardiologist could not tell for sure whether it showed normal sinus rhythm or atrial fibrillation. It showed inadequate information.
3. Dr. Stevens' interpretation that Laura Madonia was in atrial fibrillation was reasonable.
4. With a tracing like Laura Madonia's July 15, 2005 tracing, competent, careful cardiologists acting reasonably can simply disagree on the interpretation.

[24] For the detailed reasons set out below, after carefully assessing, weighing and evaluating the conflicting expert testimony on the issue of the standard of care and the July 15, 2005 electrocardiogram, I prefer the evidence of Dr. Wisenberg and, therefore, accept his evidence and reject the evidence of Dr. Baigrie.

[25] Laura Madonia had a 12 lead electrocardiogram. In interpreting the electrocardiogram, the cardiologist tries to find the relationship between the atrial activity and the ventricular activity. The electrical activity of the atrium is expressed on an electrocardiogram as a P wave and of the ventricle as a QRS complex. Dr. Wisenberg testified that there were P waves in the VI lead portion of the tracing on the electrocardiogram. P waves show normal sinus rhythm. Dr. Baigrie testified that the small (approximately 2mm by 2mm) bumps in the VI lead could be either P waves or artefact. If they are P waves it is normal sinus rhythm and if they are artefact Laura Madonia was in atrial fibrillation. Dr. Baigrie said it could be either and, therefore, that Dr. Stevens' interpretation was reasonable. Artefact is some external electrical interference, i.e. if during the test one of the wires between the patient and machine is jiggled or patient movement.

[26] Dr. Wisenberg testified that what could be seen in the VI lead was spontaneous atrial activity that Laura Madonia generated on her own and not artefact based on three factors as follows:

1. In lead VI the relationship between the P waves and the pacemaker spike before the QRS complex has a predictable precise relationship. According to Dr. Wisenberg, the probability of the relationship being so very precise would be "extremely, extremely low" if it were caused by a random artefact. The three P waves Dr. Wisenberg saw in lead VI

occurred in “a very predictable pattern immediately preceding this little pacemaker spike”. According to Dr. Wisenberg, the predictable/regular interval between the P wave, normal sinus rhythm, and the pacemaker spike in lead VI demonstrates that “the pacemaker in this case has sensed the spontaneous electrical activity of the patient, waiting the appropriate interval and then fired into the ventricle”. This, Dr. Wisenberg testified, is what the pacemaker will be set to do.

2. Dr. Wisenberg compared Laura Madonia’s May 4, 2005 electrocardiogram to the July 15, 2005 electrocardiogram. He saw the same predictable relationship between the P wave and pacemaker spike which would imply that the artefact, which is a random event, randomly occurred in precisely the same way on May 4 and July 15 in just the very same lead. Dr. Wisenberg described the likelihood of this as “extremely low”.

3. Artefact is external to the heart. Because of this, a cardiologist would expect to see the artefact in more than one simultaneously recorded leads. V1, V2 and V3 are recorded at the exact same time - if it is artefact on V1, it should be in V2 and V3 also. It is not.

[27] Dr. Wisenberg testified that these are three “compelling pieces of evidence” that the bumps in Laura Madonia’s July 15 electrocardiogram lead VI

are P waves, demonstrating normal sinus rhythm, not atrial fibrillation, and not artefact. In summarizing his opinion that these were three P waves and not artefact, he testified that:

There are three then compelling pieces of evidence that this isn't artefact, it's regular association, the fact that it's seen on other electrocardiograms and has precisely the same configuration and relationship, and that it's not seen in any simultaneously recorded lead, the probability of this being artefact is so low that I think in essence, you could discount that as any reasonable possibility that it is.

[28] Dr. Baigrie said the bumps could be either P waves or artefact, but did not testify as to the analysis as described by Dr. Wisenberg that a cardiologist would go through to determine which it is. Dr. Baigrie did not give any evidence suggesting Dr. Wisenberg's analysis was incorrect. Dr. Baigrie did not go through the analysis Dr. Wisenberg did in ruling in or out artefact. Dr. Baigrie did not in his testimony give a comparable detailed assessment or explanation as to why it might be artefact. Dr. Baigrie testified it could be one or the other and therefore that Dr. Stevens' interpretation was reasonable, without testifying as to the analysis a cardiologist would go through to distinguish one from the other. Dr. Wisenberg testified there are three compelling pieces of evidence that the bumps in lead VI are P waves and therefore that Laura Madonia had normal atrial activity, not atrial fibrillation. Dr. Baigrie did not dispute/refute these three compelling pieces of evidence in any material way. This is a significant reason why I prefer the evidence of Dr. Wisenberg.

[29] One of Dr. Baigrie's main points in his analysis that the bumps in lead VI could be either P waves or artefact was that he could not see P waves in any other leads and that if what was in lead VI were P waves he would expect to see P waves in the majority of leads. He testified that "most of the time you can see P waves in most of the leads" with normal sinus rhythm. On Laura Madonia's July 15, 2005 electrocardiogram, Dr. Baigrie could only see what might be P waves in lead VI.

[30] Dr. Wisenberg testified when people are young, a cardiologist will usually see evidence of normal atrial activity, P waves, in all leads. Dr. Wisenberg went on to testify that as people age and with disease "the sequence and the route that the electrical impulse takes through the heart may change". The electrical activity as it passes through the atria may not be seen in all the leads. According to Dr. Wisenberg, in a diseased heart the two leads where P waves are most likely to be seen are leads VI and II and you may not see them in any other leads. This is because of the particular way these two leads look at the heart. Dr. Wisenberg saw Laura Madonia's P waves in lead VI. Dr. Baigrie did not testify as to how age and disease could effect the number of leads where a cardiologist would expect to see P waves. He did not contradict Dr. Wisenberg in this regard. Dr. Wisenberg's evidence was specific to someone of Laura Madonia's age and disease. Dr. Baigrie's evidence was general. This is an additional reason I prefer the evidence of Dr. Wisenberg in regard to the July 15,

2005 electrocardiogram showing Laura Madonia in normal sinus rhythm and not in atrial fibrillation.

[31] Dr. Wisenberg testified that “to diagnose atrial fibrillation, you would have to establish that there is no evidence of atrial activity seen in any of the 12 leads”. Dr. Baigrie did not contradict this in his evidence.

[32] Dr. Baigrie testified in-Chief that in determining whether the bumps in lead VI were P waves or artefact you would look for confirmatory information on other leads. There were no P waves in the other leads. Specifically, Dr. Baigrie testified that “to make a decision on the basis of one lead, is potentially fraught with error”.

[33] Dr. Wisenberg agreed there were no P waves demonstrated in the other leads; however, Dr. Wisenberg went further in his analysis. For the reasons set out above, his opinion was that there were spontaneous P waves in lead VI, therefore Laura Madonia was not in atrial fibrillation and according to Dr. Wisenberg:

If you establish that there's sinus rhythm, not atrial fibrillation, in this part of the electrocardiogram [VI], you would have to demonstrate that there has been a complete change, if you're implying that somehow during the course of the electrocardiogram, atrial fibrillation had been present for six seconds previously, but what we see is the same sort of predictable pattern, making it very, very unlikely that the rhythm that we see here clearly, which is not atrial fibrillation, was not applied to the whole electrocardiogram, and you would interpret this electrocardiogram as showing sinus rhythm with the pacemaker firing into the ventricles, not atrial fibrillation.

According to Dr. Wisenberg, the rhythm identified in lead VI applies to the entire tracing. There is not a sudden change in the pattern “and even though you can’t see P waves in all the other leads, the pattern is similar in these 3 leads, as it is for all the others. You therefore say, the same rhythm must apply to the entire electrocardiogram, it just doesn’t apply for this three second period”. There was therefore, in Dr. Wisenberg’s opinion, further evidence in the rest of the electrocardiogram of normal sinus rhythm.

[34] Dr. Wisenberg went on to look at the rhythm strip at the bottom of the July 15 electrocardiogram. The QRS complexes occur at regular intervals and then one comes much earlier than it should according to the preceding interval. If Laura Madonia was in atrial fibrillation, Dr. Wisenberg testified that “this could not possibly occur if the pacemaker was functioning normally”. There was no evidence before me that Laura Madonia’s pacemaker was not functioning normally. Dr. Wisenberg explained in detail why these changes in interval could not happen if Laura Madonia was in atrial fibrillation. The irregular intervals in pacemaker spikes (the pacemaker occasionally firing early) could only occur if the pacemaker sensed a P wave occurring prematurely which can only happen if P waves are firing spontaneously; normal sinus rhythm, not atrial fibrillation. Dr. Wisenberg testified this is the only explanation for the premature beat having occurred. According to Dr. Wisenberg, “There can be no other explanation for it.”

Dr. Wisenberg's conclusion regarding the rhythm strip is that "this irregularity can only be explained by it responding to some spontaneous biological variation and establishes that this patient is not in atrial fibrillation at any point in this electrocardiogram, never mind that we can just see the P waves in one lead. The fact that we are seeing these premature beats scattered through the entire electrocardiogram, establishes that not only is the patient not in atrial fibrillation for these three seconds, but the patient isn't in atrial fibrillation period, during this entire recording."

[35] Dr. Baigrie did not cover this aspect of the rhythm strip in his examination in-Chief. In cross-examination he was referred to his March 21, 2007 report and his observation that the ventricular pacing is somewhat irregular and he acknowledged he had made this observation. He was then asked if he agreed with the suggestion that it was irregular because it was responding to the natural activity of the atrium firing in the appropriate manner and answered, "That's one interpretation". When asked if this would be inconsistent with atrial fibrillation, he answered "Right". When asked if this is some other evidence on the electrocardiogram of normal sinus rhythm, not atrial fibrillation, he answered, "It could be interpreted as that, yes".

[36] Dr. Wisenberg's evidence that there was confirmation of normal sinus activity in the electrocardiogram involved a much deeper analysis than Dr.

Baigrie's simplified observation that there were no P waves in any other lead. Dr. Baigrie did not contradict Dr. Wisenberg's deeper analysis. Additionally, Dr. Baigrie agreed in cross-examination, as set out above, that the rhythm strip could be interpreted as confirmatory evidence Laura Madonia was not in atrial fibrillation.

[37] Based on the above analysis, I accept the evidence of Dr. Wisenberg as to the standard of care regarding the July 15, 2005 electrocardiogram. I find that the July 15 electrocardiogram was a relatively straightforward electrocardiogram which, upon careful review, did not indicate that Laura Madonia was in atrial fibrillation. It showed she was in normal sinus rhythm. I specifically find that there were P waves in lead VI, not artefact, and that the rhythm strip showed the pacemaker firing irregularly, demonstrating that Laura Madonia was generating P waves on her own and that her atria were firing spontaneously and she was not in atrial fibrillation. Dr. Stevens misinterpreted the July 15 electrocardiogram as demonstrating that Laura Madonia was likely in atrial fibrillation. He breached the standard of care. He had a duty to carefully read the electrocardiogram as a whole and interpret it correctly and to recognize the evidence contained on it. He did not do so.

[38] The defence took the position in submissions that Dr. Wisenberg became an advocate for the Plaintiffs and that his evidence should therefore be given less

weight. The defence argues that because Dr. Wisenberg maintained that this electrocardiogram was not a difficult tracing and not one that required the exercise of medical judgment, that he was focused on proving that his interpretation was correct and did not give objective, useful evidence. I disagree.

[39] The defence also argued that because Dr. Wisenberg did not agree with Dr. Baigrie's evidence as to the frequency with which cardiologists disagree with electrocardiogram readings, that he was focused on his interpretations being correct. I again disagree. Dr. Wisenberg clarified his position clearly during his cross-examination when he testified that, "If the question is the interpretation of the rhythm, which is the case here, there is usually almost no dispute because it is not a matter of opinion what the rhythm is. The evidence is provided by the electrocardiogram." This was not his being an advocate or defending his opinion. His evidence was direct, straightforward and comprehensive. Dr. Wisenberg was firm in his testimony as to how the electrocardiogram should be read and the evidence available from it for Dr. Stevens; however, at no time did he become an advocate. He was an objective and unbiased expert.

[40] The defence also argued that the evidence of Dr. Baigrie should be preferred in part because Dr. Wisenberg has spent his career at a teaching centre hospital and Dr. Baigrie's practice was more similar to Dr. Stevens' practice than Dr. Wisenberg's. I disagree. I accept Dr. Wisenberg's evidence

that “if you are looking after patients as a cardiologist, you have to be able to identify this kind of electrocardiogram as not demonstrating atrial fibrillation whether you’re working in the community or in a university affiliated setting”. Dr. Wisenberg clarified in cross-examination that he spends 75% of his work time seeing cardiology patients and 25% doing research. Although he sees patients admitted to hospital, a great majority of his patients are outpatients seen within the physical structure of the hospital. For 27.5 out of 28 year of practicing, Dr. Wisenberg did not have residents assist him with outpatient practice. Dr. Baigrie has spent part of his career in a teaching hospital and part in office practice. I am not satisfied that the career paths of Dr. Baigrie and Dr. Wisenberg provide any basis to prefer one of their evidence over the other. Both of them have considerable experience seeing cardiology outpatients which is the nature of Dr. Stevens’ practice. There was no evidence before me that the standard of care for a cardiologist in these circumstances was different for a cardiologist working in the community or in a university affiliated setting.

[41] The defence takes the position that whether Laura Madonia was in atrial fibrillation was a question of medical judgment. Dr. Baigrie testified that it was a difficult electrocardiogram and was the type of tracing upon which competent careful cardiologists could disagree. Dr. Wisenberg was very clear that the interpretation of this electrocardiogram did not involve medical judgment. Dr. Wisenberg testified that, “The electrocardiogram is straightforward, it does not

demonstrate atrial fibrillation. It's not a judgment issue, it is a straightforward electrocardiogram which doesn't show atrial fibrillation, it shows sinus rhythm with a proper functioning pacemaker". As set out above, Dr. Wisenberg stated that it was a matter of recognizing and assessing the evidence presented by the electrocardiogram; evidence it was P waves and not artefact, and evidence of the irregular ventricular pacemaker spikes which do not occur in atrial fibrillation.

[42] Dr. Baigrie's testimony concerning the complexity of the electrocardiogram and that medical judgment was required to interpret it was superficial. A closer look at the evidence on the electrocardiogram, which evidence was explained by Dr. Wisenberg and which Dr. Baigrie did not materially contradict, demonstrated Laura Madonia was not in atrial fibrillation.

[43] Dr. Baigrie testified it could be artefact or P waves in VI. Dr. Wisenberg testified there were three pieces of evidence confirming it was not artefact. Dr. Baigrie did not materially contradict this testimony from Dr. Wisenberg. Further, Dr. Baigrie conceded under cross that the irregular ventricular pacing on the rhythm strip could be consistent with natural atrial activity. Therefore, although Dr. Baigrie said the electrocardiogram was difficult, he did not contradict Dr. Wisenberg's careful deeper analysis as to the evidence on the electrocardiogram confirming normal sinus rhythm and not atrial fibrillation.

[44] Based on my overall analysis of Dr. Baigrie's and Dr. Wisenberg's evidence as set out above, I accept Dr. Wisenberg's evidence and find that the interpretation of Laura Madonia's July 15, 2005 electrocardiogram did not require the exercise of medical judgment. I find that to an appropriately trained cardiologist, this electrocardiogram did not show a confusing picture. It needed to be looked at carefully as would be expected from any reasonably prudent cardiologist. Upon careful review, it was a straightforward electrocardiogram to assess.

[45] This finding that the interpretation of this electrocardiogram did not require the exercise of medical judgment is supported by the evidence of Dr. Stevens himself. Dr. Stevens did not testify that he found this electrocardiogram to be difficult, confounding or confusing. Nor did he testify that he considered alternative interpretations and, through a reasoning process, determined Laura Madonia was in atrial fibrillation. Dr. Stevens testified that for normal rhythm he expected to see clear evidence of P waves in leads II, III and aVF and since there were no deflections in front of the QRS complexes, this is atrial fibrillation. He said the bumps in VI were not P waves and you cannot make a diagnosis from one lead.

[46] Dr. Baigrie testified that in lead VI the bumps were either P waves or artefact. Dr. Stevens did not express in his testimony doubt as to what was in VI.

He said they were not P waves. In cross-examination he acknowledged deflections in lead VI and said they could be artefact or possibly other different arrhythmias but did not elaborate. Dr. Stevens acknowledged in cross-examination that it would be “unusual” to see artefact in one lead without artefact in other leads. Dr. Stevens gave no evidence that he went through any analysis, weighed the various possibilities, assessed the electrocardiogram as a whole and then reached his conclusion. He did not testify as to any analysis he went through to determine P waves or artefact in lead VI or any analysis he went through to consider the rhythm strip. In cross-examination he was asked if the irregular pattern in the rhythm strip could only occur with normal atria activity and he said he thought there were exceptions to this, but did not elaborate. There is no evidence he considered this evidence from the electrocardiogram in any analysis he did.

[47] Even were I to find that the electrocardiogram required the use of medical judgment, which I do not, to meet the standard of care there must be evidence that the judgment is fully applied. No such evidence was offered by Dr. Stevens. He did not explain or give any evidence as to any process he used in his review of the electrocardiogram and/or in weighing, considering and assessing the evidence it presented to justify his conclusion. I find that Dr. Stevens ignored or did not see clear and irrefutable evidence on the electrocardiogram in determining incorrectly that Laura Madonia was in atrial fibrillation. He was not

exercising medical judgment. Dr. Stevens gave no evidence that he carefully and thoroughly reviewed the entire electrocardiogram and the available evidence, and then weighed and assessed that evidence and then made a determination.

[48] During defence counsel's cross-examination of Dr. Wisenberg, counsel referred him to a August 30, 2007 report of Dr. Parker. Dr. Parker was, as I understand it, an expert retained by the Plaintiffs. Dr. Parker did not testify. Defence counsel referred Dr. Wisenberg to Dr. Parker's apparent conclusion that on at least one of the cardiograms recorded on Laura Madonia in September 2005, Dr. Parker was of the opinion that it showed Laura Madonia was in atrial fibrillation. This was referred to Dr. Wisenberg in support of the proposition that cardiologists (he and Dr. Parker) can disagree on the proper interpretation of an electrocardiogram, specifically the September 2005 electrocardiogram, and this is indicative that interpretation of electrocardiograms is an issue of medical judgment. Dr. Wisenberg was of the opinion that neither the September 26, 2005 nor September 28, 2005 electrocardiograms performed on Laura Madonia demonstrated she was in atrial fibrillation.

[49] Dr. Wisenberg responded that he was not sure what Dr. Parker was referring to, the electrocardiograms or the rhythm strips and noted that there are

numerous recordings of the electrical rhythm through Laura Madonia's entire September admission, but that if it was the electrocardiogram, they did disagree.

[50] I find this evidence of little weight to support that the interpretation of the July 15, 2005 electrocardiogram by Dr. Stevens required the exercise of medical judgment for the following reasons:

1. It relates to the September 2005 cardiogram, not the July 15 electrocardiogram.
2. Dr. Parker did not testify. Therefore, it is not entirely clear specifically what he is referring to in his August 2007 report and, therefore, I cannot determine whether he and Dr. Wisenberg agreed or disagreed. Without Dr. Parker's testimony, I cannot determine what his report referred to or why he believed Laura Madonia was in atrial fibrillation on at least one recording in September 2005.

[51] In conclusion, I accept Dr. Wisenberg's evidence as to the standard of care regarding the July 15, 2005 electrocardiogram. Dr. Stevens misinterpreted it. Laura Madonia was not in atrial fibrillation. Therefore, Dr. Stevens had no reason to prescribe Coumadin. The prescription of Coumadin was inappropriate because it was based on an incorrect interpretation of the electrocardiogram.

7. Analysis and Findings Re: Standard of Care - Dr. Stevens' Decision to

Prescribe Coumadin and Implementation of Same

[52] Dr. Wisenberg's evidence with respect to the standard of care on this issue can be summarized as follows:

1. Assuming a patient is in atrial fibrillation, a reasonably skilled cardiologist must weigh the risk of the presence of atrial fibrillation in that specific patient against the risk of initiating treatment, a blood thinning medication. This risk assessment will vary with a patient's age and underlying heart disease. It essentially involves an assessment of the risks (likelihood of blood clots/stroke/embolic events) with no treatment versus the risks of treatment. Assessing the risks of treatment would involve looking at the patient's history of bleeding problems, if any, including the pathology of the bleeding problems. If a patient has an underlying predisposition to bleeding, a cardiologist may wish to go further to establish what the current risk is by referring the patient to another expert, such as a gastroenterologist, for an assessment of the current specific risk. According to Dr. Wisenberg, if the cardiologist deemed the patient to be a higher risk individual, the cardiologist would request the gastroenterologist to do procedures "to establish that there's no structural abnormality which is predisposing this individual on an ongoing basis to bleeding after Coumadin is initiated".

2. If Dr. Stevens determined Laura Madonia was in atrial fibrillation, according to Dr. Wisenberg, Dr. Stevens should next have sat down with her and had a full discussion of the issues; “the risks that the rhythm would potentially expose her to, versus a discussion of what the risks with the treatment might expose her to, and get a sense of her understanding of those issues as to how you were explaining them to her.” Dr. Wisenberg testified that he would probably recommend to Laura Madonia, which is what he would do, is to say, “Maybe we should take a little bit of time now to determine for you, what the specific risks are for treatment with Coumadin if we’re thinking of initiating it. Because of all your prior problems that you’ve had with GI bleeding in the past, maybe we should just go that extra step further.”

3. Dr. Wisenberg testified that he would expect this discussion to be with the patient in the office. He testified that a doctor gets a better sense when he/she speaks in person to the individual, through body language and eye contact, of the patient’s understanding of the issues and the patient’s acknowledgement they have understood. Dr. Wisenberg testified “it’s much better done in person”.

4. Dr. Wisenberg testified that the six weeks that elapsed between the July 15th, 2005 electrocardiogram and the September 6th, 2005 prescription

of Coumadin gave the opportunity for face-to-face discussions between Dr. Stevens and Laura Madonia and the opportunity for a gastrointestinal consultation and whatever procedures might have been deemed necessary to define the current risk of bleeding before Coumadin was initiated.

5. In Dr. Wisenberg's opinion, a prudent cardiologist would have referred Laura Madonia for a gastroenterology consult to assess the current state of risk of bleeding based on several occurrences in Laura Madonia's past that had established a risk of bleeding and that her chart had documented that she had been told in the past by someone not to take anti-coagulation therapy again.

6. Dr. Wisenberg testified that a cardiologist starting a patient on Coumadin must make sure that the patient understands the "critical importance" of having blood tests (the I.N.R. tests) done at the frequency set out by the cardiologist. A cardiologist should have this discussion with the patient in a setting where the cardiologist is "very sure" that the patient understands the importance of frequent monitoring. In an office visit, the cardiologist can write it down for the patient. Dr. Wisenberg's opinion was that given that Laura Madonia was at a higher risk of bleeding, her therapeutic range for the I.N.R. level was better to be between 2 and 2.5.

[53] Dr. Baigrie's evidence with respect to the standard of care on this issue can be summarized as follows:

1. Dr. Baigrie testified that if Dr. Stevens believed that the July 15, 2005 electrocardiogram represented something other than normal sinus rhythm, such as atrial fibrillation, it was reasonable for Dr. Stevens to prescribe Coumadin given Laura Madonia's other cardiovascular issues and particularly her large left atrium. Atrial fibrillation would increase Laura Madonia's already existing risk of stroke. Thinning the blood with a blood thinner like Coumadin is a standardized way of trying to minimize the risk of stroke.
2. Dr. Baigrie stated that the INR test is a worldwide standardized test to measure the degree of blood thinning created by Coumadin.
3. In terms of whether it was appropriate for Dr. Stevens to have had the discussion with Laura Madonia about prescribing Coumadin by telephone, Dr. Baigrie testified that ordinarily that discussion would take place in an office. He testified that he didn't see a problem with Dr. Stevens having this discussion with Laura Madonia over the phone because Dr. Stevens had been following her for some years, he had a history with her and she was acquainted with Coumadin and the need to monitor and Dr. Stevens was reiterating what had probably been gone

through at least once before by others. Dr. Baigrie referenced that Laura Madonia's records from Georgia indicate that from approximately December of 1999 to May of 2000 Laura Madonia had been placed on Coumadin when she had a retinal embolism. Her records indicate that she had a series of INR tests. Dr. Baigrie also referenced that, to his knowledge, Mr. Madonia had also been on Coumadin for many years. Dr. Baigrie concluded that it was reasonable for Dr. Stevens in these specific circumstances to have had the discussion regarding prescribing Coumadin with Laura Madonia over the phone.

4. Dr. Baigrie testified that the monitoring of Coumadin with INR testing is extremely important. INR testing is, according to Dr. Baigrie, "an absolutely necessary part of the prescribing of Coumadin, no matter how long one takes it or how much one takes. It's a package. If you're going to take Coumadin, you're going to get INR's." Dr. Baigrie testified that Coumadin invariably equals INR testing. Dr. Baigrie confirmed in cross-examination that it is important that the patient understand the need for monitoring and the schedule of monitoring.

5. Dr. Baigrie summarized that Dr. Stevens met the standard reasonably to be expected of a cardiologist practicing in Ontario in 2005 in

his decision to prescribe Coumadin for Laura Madonia and in the manner in which he communicated it.

6. Dr. Baigrie acknowledged in cross-examination that Laura Madonia being in atrial fibrillation had profound implications for her health and that there had to be a weighing of the risks and an explanation of the risks to Laura Madonia. Dr. Baigrie agreed in cross-examination this was a new diagnosis, “a potentially serious diagnosis, a potentially serious form of therapy which requires some thought” by both Laura Madonia and Dr. Stevens.

7. Dr. Baigrie confirmed in cross-examination that a weighing of Laura Madonia’s risks involved consideration of Laura Madonia’s gastrointestinal bleeding history. Dr. Baigrie testified that a gastroenterologist would be in a better position to tell what a patient’s gastric pathology is, but not necessarily in a better position to tell if the patient was going to bleed or not with Coumadin unless a specific diagnosis like duodenal ulcer was active at that time. He said some cardiologists would have referred Laura Madonia to a gastroenterologist and some wouldn’t have. Dr. Baigrie agreed that when embarking on a potentially dangerous course of treatment it is prudent to get as much information as possible and that an assessment by a gastroenterologist is reasonable.

[54] Dr. Stevens' evidence as to his decision to prescribe Coumadin to Laura Madonia and his discussion with her as to the risks and his discussions with her as to monitoring can be summarized as follows:

1. Dr. Stevens assessed Laura Madonia's risk of stroke as high given his diagnosis that Laura Madonia was in atrial fibrillation, her age, her diagnosis of hypertrophic cardiomyopathy, her previous retinal embolism, the presence of mitral annular calcification and her enlarged atrium. He explained in his evidence how these factors increased Laura Madonia's risk of stroke given his diagnosis of atrial fibrillation.
2. In examination in-Chief Dr. Stevens testified that in weighing the risk of or benefits of prescribing or not prescribing Coumadin for Laura Madonia, he considered the risk "that she had the previous history of bleeding and she had the partial gastrectomy as a result of that, so that was the reason why she had been told not to take Coumadin previously". With respect to bleeding, Dr. Stevens also referred to 2003 when Laura Madonia attended at a hospital in Florida and was diagnosed with a gastric bezoar. Dr. Stevens testified that in 2003 there was no evidence of gastric bleeding and that one of the medications Laura Madonia received at the Florida hospital was low-dose Lovenox which Dr. Stevens testified is a blood thinner. To Dr. Stevens' knowledge, Laura Madonia had not

suffered from any bleeding at the time of the gastric bezoar or with the prescription of Lovenox.

3. Dr. Stevens testified that he felt Laura Madonia's risk of stroke was high. Even though she had a history of bleeding, he was reassured by the investigation/hospital's actions in 2003 and that there had been no bleeding then or since. She had been challenged in 2003 with the blood thinner Lovenox with no bleeding. Considering all of these factors, he prescribed Coumadin.

4. Dr. Stevens testified that he had a phone conversation with Laura Madonia on September 6, 2005 with respect to initiating Coumadin. He testified that they discussed the following:

- i) He told her that she was in atrial fibrillation and that he thought that they should consider putting her on Coumadin because of the risk of rhythm increasing the risk of stroke.
- ii) Dr. Stevens told Laura Madonia that he thought she should come into the office to discuss this particular issue and that Laura Madonia didn't really want to come into the office for that purpose.

iii) They discussed that she had bleeding issues before and that there had been concern in terms of the Coumadin previously.

iv) With respect to the bleeding issue, Dr. Stevens recalled discussing with Laura Madonia that more recently the tests had been very reassuring.

v) Laura Madonia indicated that she would proceed with Coumadin without coming into the office, and that Mr. Madonia had been on Coumadin for many years and they were comfortable to start Coumadin without coming in to specifically discuss it.

vi) Dr. Stevens told Laura Madonia she would need monitoring (blood work) in a few days and again after that within a week of so, and then on a weekly basis. Laura Madonia indicated she understood.

vii) Laura Madonia, in Dr. Stevens' view, understood the discussion relating to the risk of bleeding versus the risk of stroke, and the need for monitoring with blood tests.

5. Dr. Stevens testified that when he reviewed the INR test results for blood drawn September 9, 2005, he increased the Coumadin dosage to move Laura Madonia towards the therapeutic range for the drug. His assistant, Joann Rasmussen, communicated with the Madonia's thereafter.

6. Dr. Stevens said that it is standard in his practice that when advising a patient of a new dosage of Coumadin, his office would indicate to the patient when the next blood work should be done and that he would never increase a dosage without setting further monitoring.

7. Dr. Stevens testified that Laura Madonia was generally a compliant patient. She would follow his instructions and not miss appointments. Occasionally Laura Madonia would not take her diuretic medication if she had a social engagement. On these occasions, she would sometimes change the timing of the diuretic and reduce the dose on her own, otherwise she was compliant with medication. Dr. Stevens agreed Laura Madonia was generally a very motivated patient interested in her own health.

8. Dr. Stevens testified that he made a "late entry" on December 15, 2005 (after Laura Madonia's death) on Laura Madonia's anticoagulation therapy sheet in which he attempted to clarify when Laura Madonia was

instructed by Joann Rasmussen to next obtain an INR test and that his interpretation was incorrect and that he had misread the sheet.

[55] Joann Rasmussen is a medical secretary who has worked for Dr. Stevens for just over 4 years. Her testimony with respect to the prescribing of Coumadin and INR monitoring to Laura Madonia can be summarized as follows:

1. Ms. Rasmussen has no independent distinct recollection of her communications with Laura Madonia on September 6, 2005. She testified that her usual practice would have been to have a discussion with the patient starting Coumadin about the need for blood work, which lab she would like the patient to go to, to take the Coumadin in the evening and for the patient to call the office if he or she do not hear from the office after the blood work. She testified that she would have communicated this to Laura Madonia by phone on September 6, 2005.

2. Ms. Rasmussen identified that she completed the Anticoagulant Therapy Sheet for Laura Madonia (except for the late entry). From this note and without independent recollection, she testified that:

- 1) She would have made the September 6, 2005 entry when Dr. Stevens told her Laura Madonia was starting on Coumadin and the dosage.
- 2) Laura Madonia had her first INR blood test on September 9 and her level was 1.1.
- 3) She would have taken the report from the lab, entered the date of the test and result on the sheet and showed it to Dr. Stevens for his instructions.
- 4) Looking at the sheet, the instructions from Dr. Stevens were 2.5 mg alternating with 5 mg daily.
- 5) Looking at the sheet, Ms. Rasmussen testified that Dr. Stevens would have asked her to tell Laura Madonia to recheck her blood work on Monday.
- 6) Looking at the sheet, Ms. Rasmussen testified that she called September 13. She had no doubt she called either Laura Madonia or Mr. Madonia on that date, but had no distinct recollection of having done so or what she said or who she spoke to. She made no note of who she spoke to. In cross-examination, she acknowledged that nothing prevented

her from making a note of who she spoke to and precisely the instructions given. She testified it was not possible that she advised of the dose increase, but failed to mention the blood recheck date; however, she acknowledged that she had no distinct memory of the instructions given.

7) She had never encountered while working for Dr. Stevens a situation which a dose of Coumadin had been increased without a specified date given for monitoring.

8) Ms. Rasmussen testified that Dr. Stevens' late entry on the sheet did not correctly interpret the data she had written on it.

9) Mr. Rasmussen acknowledged in cross-examination that it is essential that anyone looking at the sheet would know when the blood work was to be done next and that it is best that the sheet eliminate any confusion.

10) Ms. Rasmussen testified in cross-examination that she did not recall Dr. Stevens consulting with her before he made the late entry on the sheet and that if he wanted to make an accurate late entry, he should have spoken to her.

[56] Mr. Madonia's testimony with respect to the implementation of the Coumadin therapy can be summarized as follows:

1. Dr. Stevens spoke by telephone with Laura Madonia on September 6, 2005. Mr. Madonia listened only to Laura Madonia's side of the conversation. Laura Madonia reported to her husband that she was to start on Coumadin today and have INR testing in 3 days. He acknowledges there must have been a follow-up call from Dr. Stevens' office about which pharmacy and which lab to go to. He went to the pharmacy to pick up the Coumadin and the prescription label said $\frac{1}{2}$ tablet daily (2.5 mg).
2. On the 9th of September Mr. Madonia took Laura Madonia to the MDS lab for blood testing (INR).
3. A woman from Dr. Stevens' office called on either the 13th or 14th of September and spoke to Mr. Madonia. Mr. Madonia testified that she told him the INR was 1.1 and that Dr. Stevens wanted Laura Madonia's dose increased to a full tablet one day (5 mg) and a half tablet the next day (2.5 mg). He did not receive further instructions and specifically he testified that he absolutely did not receive any instructions about further blood tests (INR). Mr. Madonia assumed Dr. Stevens would tell them when to get the next blood test. Mr.

Madonia is on Coumadin and testified that he gets his blood tested once per month.

[57] After considering the standard of care evidence of both Dr. Wisenberg and Dr. Baigrie and having considered the other evidence summarized above, I find that Dr. Stevens fell below the standard of care in the implementation of Coumadin therapy for Laura Madonia in the following ways, each of which will be analyzed in detail below:

1. In assessing Laura Madonia's risk of bleeding if put on Coumadin and his discussion of said risk with Laura Madonia, Dr. Stevens failed to consider that Laura Madonia had been on anticoagulation medication in 1999-2000 at the time of her retinal embolism and suffered significant bleeding from the GI tract and the medication was stopped and she had been told not to take blood thinners again.
2. In assessing Laura Madonia's current risk of bleeding if put on Coumadin and in his discussions with Laura Madonia of said risks, Dr. Stevens did not consider, nor discuss with Laura Madonia the possibility of having Laura Madonia's current gastric pathology assessed by a gastroenterologist. Further, he did not recommend this to her.

3. Dr. Stevens and his staff did not convey to Laura Madonia in a clear manner when her next INR testing was to occur. This is specifically with respect to the second testing.

[58] The Plaintiffs have raised other aspects of Dr. Stevens implementation of Coumadin therapy, such as Dr. Stevens' delay and that the discussions of risks/benefits was done in a telephone call instead of an office visit, and for the reasons set out below, in the particular facts of this case, I do not find these necessary considerations. Whether or not Ms. Madonia refused to come into the office or not is also irrelevant given the facts of this case. Dr. Stevens fell below the standard of care regardless of whether Laura Madonia came into the office or not. The standard of care was the same whether the communications were over the phone or in person.

[59] Both Dr. Wisenberg and Dr. Baigrie agreed that if Dr. Stevens believed Laura Madonia was in atrial fibrillation the standard of care required Dr. Stevens to consider the risks of stroke versus the risks of anticoagulation therapy and that part of this risk assessment would involve consideration of her bleeding history. Dr. Stevens was required to consider these aspects and discuss them with Laura Madonia. As set out above, I find that Dr. Stevens failed to consider or discuss with Laura Madonia that she had been on anticoagulation medication in 1999-2000 at the time of her retinal embolism and suffered significant bleeding from

the GI tract and the medication was stopped and she was told never to take anticoagulation therapy again. Dr. Stevens was clearly told this by Laura Madonia. In Dr. Stevens' note to Dr. Graham (Laura Madonia's family physician) dated December 5, 2000 dictated by Dr. Stevens after his initial consult with Laura Madonia he wrote "She apparently had a retinal embolism previously and was placed on anticoagulation. However she had significant bleeding from the GI tract and this was stopped. She had been told not to take blood thinners again."

[60] Dr. Stevens' note of his telephone conversation with Laura Madonia of September 6, 2005 makes no reference to him discussing with Laura Madonia her previous experience of bleeding while on anticoagulation medication and that she had been told not to take it again. The note refers to a prior history of GI bleeds, but no reference to bleeds while on anticoagulation therapy. It is clear from Dr. Stevens' oral testimony (in-Chief and cross) that Dr. Stevens, by prior history of GI bleeding, was referring to Laura Madonia's bleeding she had when she had watermelon stomach in the mid 1990's and not the bleeding while she was on anticoagulation therapy in 1999-2000 as a result of the retinal embolism.

[61] A careful review of Dr. Stevens' testimony confirms that Dr. Stevens equated Laura Madonia being taken off Coumadin in the past to bleeding she had at the time of her watermelon stomach and not because she specifically had

bleeding while on Coumadin. This is evident in both his examination in-Chief and cross-examination as follows:

1. Dr. Stevens testified in-Chief in response to a question about his letter to Dr. Wigle dated August 31, 2005, "Well, the risk was that she had the previous history of bleeding and she had the partial gastrectomy as a result of that, so that was the reason why she had been told not to take Coumadin previously". He equated the warning she had been given not to take Coumadin to her gastrectomy (related to her watermelon stomach) which was clearly wrong. Her gastrectomy was in the mid 1990's. She had been on an anticoagulant in 1999-2000 as a result of a retinal embolism and had had significant GI bleeding and the medication was stopped. This is when and why she was told not to take anticoagulants again. From Dr. Stevens' testimony it is clear that he did not accurately recall Laura Madonia's medical history in this regard. Dr. Stevens understood she had been given anticoagulation therapy in 1999-2000 and been told to stop because of her previous bleeding with the watermelon stomach. In fact, she was told to stop because she bled on Coumadin. Dr. Stevens never referred to this bleeding while on Coumadin in his examination in-Chief and, as set out below, initially denied knowledge of this in cross-examination until confronted by his own initial letter of

December 5, 2000 which summarized what he had been told by Laura Madonia of her own medical history.

2. As set out above, Dr. Stevens' note of September 6, 2005 does not document any discussion he had with Laura Madonia about her previous experience of significant bleeding while on anticoagulation medication. In his oral evidence in-Chief he said, "...we did discuss the issue of the fact that she had had bleeding issues before and there had been concern in terms of the Coumadin previously". He never testified that he discussed with her that she had Coumadin in 1999-2000 and had significant bleeding as a result. Once again, he appears to equate the bleeding with her mid 1990's watermelon stomach and related surgery and not bleeding she had on the very drug he was prescribing.

3. That Dr. Stevens did not discuss with Laura Madonia in September of 2005 the risks and implications of what he had been told by Laura Madonia in December 2000 about her bleeding while on anticoagulation therapy is absolutely clear from his answers in cross-examination, particularly the following question and answer sequence:

Q. And you would have known, and in fact you knew that Mrs. Madonia suffered from gastric intestinal bleeding as a consequence of that anticoagulation therapy, right?

A. At that particular time?

Q. When it was being administered to her, yes, in 2000?

A. I'm not sure I knew that at the time

Q. Well let me look -- if you'd just turn to your very first note to Dr. Graham, page one, Doctor? Would you look, please, at the -- I guess the second main paragraph, it is the third sentence from the end of that paragraph, "She apparently --" do you have it there?

A. Yes, I do.

THE COURT: I haven't found it. I'm on the letter, but I can't find...

MR. MACDONALD: If you'd go to the -- if you'd just take that big fat main -- do you see it there?

THE COURT: Yes, I see it now.

MR. MACDONALD: You see it now?

BY MR. MACDONALD: "She apparently had a retinal embolism previously and was placed on anticoagulation." So we know that, and you knew, in December of 2000, that she had had a retinal embolism and required anticoagulation therapy, correct?

A. Yes.

Q. Yes? And then you go on to say, "however, she had significant bleeding from the gastro intestinal tract and this was stopped," right?

A. Yes.

Q. You recorded that and then you go on to say that, "She has been told not to take blood thinners again," right?

A. Yes.

Q. So that one of the very first things you knew about Mrs. Madonia, was that she had already been on anticoagulation therapy and that that very therapy had resulted in a gastro intestinal bleed that led to stopping that therapy, right?

A. Yes.

Q. And that we can only presume that the type of therapy that was being implemented, would result in INR values of between two and three, correct?

A. That's right.

Q. And so that she had the experience and the history of bleeding even with these purported therapeutic values of between two and three, right?

A. I'm not sure I was told of actual bleeding related to that episode of Coumadin.

Q. Well Doctor, you're the one who wrote this note. These are your words. This reflects your understanding of what her history was, correct?

A. Yes.

Q. And to go over it again, "However, she had significant bleeding from the GI tract and this was stopped," and I suggest to you, you meant the anticoagulation therapy was stopped?

A. Yeah, I believe that I understood that she had a history of GI bleeding before and was placed on the Coumadin, and then afterwards the Coumadin was stopped because of the previous history of bleeding.

Q. No, that's not what you've written, Doctor, and that's not what happened. What you've written, Doctor, is that she stopped because she had bleeding as a result of the Coumadin. That's what you've written, that's what she's told you and that's what you understood, right?

A. Okay.

Q. Yes?

A. Yes.

Dr. Stevens maintained initially that the Coumadin was stopped in 2000 because of previous history of bleeding and ultimately admitted that his December 5, 2000 letter clearly indicates otherwise. He did not therefore discuss with Laura Madonia in September of 2005 in reaching the decision to implement Coumadin that she had experienced significant bleeding 5 years earlier while on the same therapy.

4. Dr. Stevens wrote a letter to Dr. Wigle dated August 31, 2005 concerning Laura Madonia. Dr. Wigle, according to Dr. Stevens, is a leading expert in hypertrophic cardiomyopathy and Dr. Stevens was seeking a second opinion. In cross-examination, Dr. Stevens acknowledged he did not state in the letter that Laura Madonia had a history of gastric bleeding and previously had bleeding as a consequence of anti-coagulation. Dr. Stevens acknowledged that this was a pertinent part of her history that Dr. Wigle ought to have known about. Dr. Stevens stated that his December 2000 initial consultation letter would have gone with the referral letter to Dr. Wigle, yet acknowledged that there is no record in his file that the December 2000 note was sent to Dr. Wigle. I find that Dr. Stevens did not put this in his letter to Dr. Wigle because he had forgotten about his note of December 5, 2000 or had misrecalled its content which is clear from his cross-examination referenced above.

[62] In assessing Laura Madonia's risk of bleeding if Coumadin therapy was implemented, Dr. Stevens did not consider Laura Madonia's previous history with anticoagulation therapy and that she had significant bleeding when on said therapy and had been told not to take it again, and he did not discuss this important aspect of her risk assessment with her. This fell below the standard of care.

[63] With respect to the finding that Dr. Stevens fell below the standard of care in not considering or discussing with Laura Madonia a referral to a gastroenterologist, Dr. Wisenberg testified that a prudent cardiologist would have referred Laura Madonia to a gastroenterologist. Dr. Baigrie's evidence on cross-examination on this issue is as follows:

Q. And the weighing of the risks would involve the consideration of the implications for this patient, given her gastrointestinal bleeding history, right?

A. Right.

Q. And that that would likely be best dealt with by a specialist in gastroenterology, right?

A. What would be best dealt with? The assessment of the risk?

Q. Yes, as far as this patient's history of gastro intestinal bleeding? A gastroenterologist is in a better position to assess the risk than would be a cardiologist, agreed?

A. You know, I'm not sure I know the answer to that. A gastroenterologist would be in a better position to tell you what the patient's gastric pathology is. I'm not sure that they're in a better position to tell you whether the patient is going to bleed or not, unless a specific diagnosis like duodenal ulcer or something were active at the time.

Q. Or how about watermelon stomach?

A. Well, that was a problem five or ten or 15 years ago, and the patient had an operation at the time. I think it was a partial gastrectomy. The patient had a subsequent operation where the stomach was opened and explored and a

gastric ...(inaudible) ... I believe was removed, and the stomach was inspected and no further removal of stomach was done at that time, so one could assume that the watermelon stomach, which is not the whole stomach, but focal area of the stomach, perhaps had been looked after -- taken care of, removed.

Q. Well you'd want to be sure of that though, wouldn't you? You'd want to be sure that that wasn't a risk, so the best way to ensure that that wasn't a risk, would be to suggest that the patient see a gastroenterologist just to confirm that, right?

A. Some would do that, some wouldn't. If the patient hasn't bled for seven or eight years and a patient has had no evidence of any active bleeding, I think you've got to weigh that against whatever the gastroenterologist finds, I'm dealing with what I think is a very high probability of embolism stroke.

Q. But Doctor, it's only prudent to explore and expand the available information to you as a physician when you're embarking upon a potentially dangerous course of treatment, right?

A. Yes.

Q. And in those circumstances it's prudent to get as much information and knowledge as you can, right?

A. Yes.

Q. And if it's not unreasonable, then an assessment by a gastroenterologist is appropriate, right?

A. It's reasonable.

[64] Dr. Baigrie agreed that an assessment by a gastroenterologist was reasonable. There was no real disagreement between Dr. Wisenberg and Dr. Baigrie about the reasonableness of such a referral, although Dr. Baigrie did not say that not referring Laura Madonia fell below the standard of care.

[65] It is clear that Dr. Stevens never even considered a referral to a gastroenterologist between July 15, 2005 and his discussion with Laura Madonia on September 6, 2005. He did not testify that he did and his notes of his conversation with Laura Madonia on September 6, 2005 do not reference that he had any discussions with Laura Madonia about this. Dr. Stevens should have considered such a referral and discussed this “reasonable” (according to Dr. Baigrie) course of action with Laura Madonia. Dr. Stevens in cross-examination testified that referring Laura Madonia to a gastroenterologist was an option and possibility and that there was no downside to his doing so. He felt, however, that he had enough information to proceed without such a referral, including information that she had had investigations in Florida in 2003 related to her gastric bezoar and that said investigations did not show a clear source of bleeding at that time and that Laura Madonia had not reported to him any GI problems in May or July of 2005. Unfortunately, Dr. Stevens’ belief that he had enough information to go ahead without such a referral was rooted in his incorrect understanding that the discontinuation of Coumadin in 1999-2000 was related to her watermelon stomach/gastrectomy in the mid 1990’s and not to

significant bleeding she had in 1999-2000 related to her taking Coumadin. Dr. Stevens fell below the standard of care in not considering or discussing with Laura Madonia a referral to a gastroenterologist. A gastroenterological consultation could have been done in the 6 to 7 weeks between July 15, 2005 and September 6, 2005 according to Dr. Wisenberg and neither Dr. Baigrie nor Stevens gave any evidence to the contrary.

[66] Dr. Stevens and his staff did not convey to Laura Madonia in a clear manner when her next INR testing would be after her initial testing on September 9, 2005. Both Dr. Wisenberg and Dr. Baigrie agree that the standard of care requires a clear communication of the INR testing regime to a patient taking Coumadin. Dr. Stevens did not do so.

[67] With respect to the communication on September 6, 2005, Laura Madonia understood that she was to be tested in 3 days, as Mr. Madonia testified. She presented herself to the lab on precisely the third day. The prescription Dr. Stevens wrote for Coumadin said "INR 3 days". Laura Madonia did exactly what the prescription said.

[68] With respect to the communication of the next scheduled test, Mr. Madonia testified that this was absolutely not communicated to him in his telephone conversation with someone from Dr. Stevens' office on September 13th or 14th. Joanne Rasmussen had no specific recollection of the conversation or

even who she had it with, but based on the anticoagulation therapy sheet testified that she told someone on September 13, 2005 that the next testing was to be on the Monday [this would have been September 19th].

[69] Based on the following, I accept the evidence of Mr. Madonia and reject the evidence of Joann Rasmussen:

1. Mr. Madonia had a specific definitive recollection and Joann Rasmussen did not.
2. When Laura Madonia was told initially to go for testing in 3 days, she did. If Mr. Madonia was told on September 13 to go on the 19th, they would have. She had just demonstrated her compliance by going on the 9th and, according to Dr. Stevens, had been a compliant patient except occasionally with respect to her diuretics.
3. Joann Rasmussen was relying on the anticoagulation therapy sheet. While deference would generally be given to a written record, there are several problems with this record that make it unreliable. First, Ms. Rasmussen did not make any note of who she spoke to or what specific instructions were given about re-testing the INR level, for example, the record does not indicate the date on which the next testing was to occur. Secondly, Ms. Rasmussen relies on her notes on a sheet that even Dr.

Stevens misinterpreted. The sheet is not supported by any other documentation in the chart. Dr. Stevens made his late entry on the anticoagulation therapy sheet on December 15, 2005 after he knew of Laura Madonia's death and after he had received a letter from the Plaintiffs' counsel. He wrote, "the above reads 're-check Mon Sept 13,' four days after dose increase." This was clearly wrong. September 13, 2005 was not a Monday. The Monday following September 13 was September 19th which was six days after September 13th. If the sheet was confusing to Dr. Stevens, it cannot possibly be relied on as it was his record, a record he was responsible for. If the chart was clear, Dr. Stevens would not have had to add this last entry to elaborate. Ms. Rasmussen had no independent recollection. She was relying on a note not even Dr. Stevens understood and whatever practice she had. Her evidence must be rejected on this point.

[70] Dr. Stevens acknowledged that there was a 6 to 7 week delay in his telling Laura Madonia that, in his view, Laura Madonia was in atrial fibrillation and gave no real explanation for this delay. Delay of this nature may fall below the standard of care, but it is not necessary for me to decide this issue in this case because the delay actually delayed Laura Madonia starting the unnecessary Coumadin therapy.

[71] Similarly, whether the discussions between Dr. Stevens and Laura Madonia should have taken place in person in Dr. Stevens' office or over the phone is not a necessary determination because regardless of how the discussions took place, the substance of the discussion itself fell below the standard of care in the manner I have referred to above. My findings also make irrelevant the issue of whether Laura Madonia refused to come into the office or not. Whether the discussion took place on the phone or in person, it had to meet the same standard of care in terms of the completeness of the discussion.

[72] In terms of Dr. Stevens' decision to implement Coumadin therapy for Laura Madonia, he testified that he drew comfort from the letter he received back from Dr. Wigle dated September 13, 2005 which said in part, "She certainly does require anticoagulant therapy with Coumadin. I would be very forceful about that." Dr. Wigle did not testify and therefore it is impossible for me to know what information he based his recommendation on. Although Dr. Stevens had said in his letter referring Laura Madonia to Dr. Wigle dated August 31, 2005 that he expected Laura Madonia would not be keen on full anticoagulation he never explained in his letter to Dr. Wigle the basis for this. Specifically, he did not reference her previous bleeding when on Coumadin or that she had been told not to take it again. Further, Dr. Stevens' letter did not refer to Laura Madonia's history of watermelon stomach with the gastrectomy. Dr. Stevens testified that he thinks his initial letter of December 5, 2000 to Laura Madonia's family doctor

was sent to Dr. Wigle, but there is no proof of this and certainly no mention of this history of bleeding with Coumadin and with watermelon stomach in Dr. Wigle's letter. At the time of writing the letter, Dr. Wigle had not seen Laura Madonia. Dr. Stevens does not know when he received Dr. Wigle's letter, whether it was before or after Laura Madonia's death. For these reasons, I can place no reliance on Dr. Wigle's letter of September 13, 2005 as to whether Dr. Stevens' implementation of Coumadin therapy fell below the standard of care.

8. Causation

(a) The Law

[73] The current law on causation is summarized by the Ontario Court of Appeal in *Monks v. ING Insurance Company of Canada*, 2008 ONCA 269 in paras. 85 and 86 as follows:

[85] *Athey v. Leonatii*, *supra*, is the leading Canadian case on causation in tort law. In *Athey*, Major J. reiterated the following well-established principles:

(1) The general, but not conclusive, test for proof of causation is the "but for" test, which requires a plaintiff to show that his or her injury would not have occurred but for the negligence of the defendant (para. 14).

(2) In certain circumstances, where the "but for" test is unworkable, causation may also be established where it is demonstrated that the defendant's negligence "materially contributed" to the occurrence of the tort victim's injury. It is not necessary for the plaintiff to establish that the defendant's negligence was the sole cause of the injury (paras. 15 and 17).

(3) Liability will be imposed on a defendant for injuries caused or materially contributed to by his or her negligence. That liability is not reduced by the existence of other non-tortious contributing causes (paras. 22 and 23). [Citations omitted.]

[86] More recently, in *Resurfice Corp v. Hanke*, [2007] 1 S.C.R. 333, the Supreme Court of Canada clarified the exceptions to the “but for” causation test and the circumstances in which the material contribution test may be applied. I do not understand *Resurfice* to alter the basic causation principles that I have described.

(b) Analysis and Findings re: Relevant Evidence

[74] I find that the Plaintiffs have proved on a balance of probabilities that the failures of Dr. Stevens to meet the standard of care caused or materially contributed to Laura Madonia’s death.

[75] I accept Dr. Wisenberg’s evidence on causation which can be summarized as follows:

1. The risk of bleeding is directly proportional to how high the INR level is. The higher the number, the less likely the blood will clot. A number of 6.7 (which was Laura Madonia’s INR level on her admission to hospital September 26, 2005) is extremely high and will potentially be made worse with any underlying cause of bleeding.
2. With Laura Madonia’s history of gastrointestinal bleeding, her prior gastric operation to reduce her risk of bleeding, bleeding occurring after that, despite the surgery and an attempt to treat her retinal embolism with

Coumadin that failed because of bleeding, her current risk of bleeding was increased with that high of an INR level.

3. It was Dr. Wisenberg's opinion that it was reasonable for an endoscopy to be performed on Laura Madonia upon her admission to hospital on September 26, 2005 to establish the site of the bleeding that was occurring and how it might best be managed.

4. Dr. Wisenberg explained "The drug Coumadin was initiated, ultimately producing an extremely high INR level, and in the setting of what appears to have been pre-existent pathology based on the multiple small ulcerations identified at the time of the endoscopy performed on September 26th, ultimately led to the fatal gastrointestinal bleed which she had. The bleed itself was not fatal, but lead to the endoscopy being performed in response to the situation created by it and, as a complication of the endoscopy, she suffered irreversible brain damage.

[76] Dr. Baigrie did not touch on causation as in his view both the diagnosis of Laura Madonia as being likely in atrial fibrillation and Dr. Stevens' subsequent actions implementing Coumadin therapy met the standard of care.

[77] Dr. Stevens failed to correctly read and interpret the electrocardiogram. Laura Madonia was not in atrial fibrillation. Dr. Stevens acknowledged in his

discovery read into evidence that if Laura Madonia was not in atrial fibrillation, there would not have been need for him to have implemented the Coumadin therapy. Dr. Wisenberg testified that there was no need for Laura Madonia to be on Coumadin as she was not in atrial fibrillation. Had she not been prescribed Coumadin, she would not have developed the severe gastrointestinal bleeding which led her to seek medical attention on September 26, 2005. The cause of her bleeding was clearly Coumadin. Her INR level was 6.7, well above the therapeutic range, and but for the Coumadin, her INR would not have reached this level. As a consequence of the bleed, she was taken to an endoscopy lab. During the course of the procedure, she was given a sedative/anaesthetic Propofol. According to the evidence of Dr. Wisenberg, "As a result of that, her blood pressure fell markedly and there was inadequate perfusion to the brain, and although she was able to be resuscitated from that situation, the length of time that her brain had not received oxygen, effectively led to her brain drying, so although the rest of her body had been resuscitated, her brain had been injured beyond the point of salvage." The hospital records reveal that after discussion with the hospital staff, the family decided to withdraw life support and Laura Madonia died on October 2, 2005.

[78] The Plaintiffs have met the "but for" test.

[79] But for Dr. Stevens' incorrect reading of the electrocardiogram, Laura Madonia would not have been given Coumadin. But for Dr. Stevens' failure to advise Laura Madonia of when her next INR test (the second) was to be done, her INR would not have reached 6.7 and she would not have bled. But for the bleeding, there would have been no need for the endoscopy and no need for the Propofol.

[80] Counsel for Dr. Stevens argued that the causal link was broken on September 16, 2005 when Laura Madonia attended at Joseph Brant Hospital with complaints of epigastric pain and difficulty swallowing, and did not tell anyone that she was on Coumadin. The records of Joseph Brant and the evidence of Mr. Madonia are clear that the Madonias were asked what medication Laura Madonia was on. Mr. Madonia testified that they provided a typed list of medication. Coumadin was not on the list. The list had not been updated. Mr. Madonia confirmed that they did not tell the Hospital Laura Madonia was on Coumadin. In his testimony, Mr. Madonia said he did not tell anyone because they were there about complaints that had nothing to do with Laura Madonia's heart.

[81] Counsel for Dr. Stevens' position is that if the Madonias had told the emergency department about Laura Madonia being on Coumadin, an INR test could have been done. If there was concern about her levels, it could have been

stopped just as it had been in 2000. According to the Defendant, this failure to communicate that Laura Madonia was on Coumadin was a negligent act for which Dr. Stevens is not liable, which was the true cause of her death and the apportionment of liability to Dr. Stevens should be zero. I do not accept this position. Although from all of the evidence, it is very likely that if the Madonias had told the Hospital, an INR test could have been taken, I have no evidence of any kind as to what Laura Madonia's INR level would have been on September 16, 2005. As Dr. Wisenberg testified, it would only be guess. This cannot prove the evidentiary foundation for contributory negligence.

[82] Further, according to Dr. Stevens' and Ms. Rasmussen's testimony, the instructions, which I found had not been communicated to the Madonias, were that Laura Madonia's next INR test would be Monday September 19, 2005, several days after the hospital attendance on September 16, 2005. It is disingenuous to suggest that Laura Madonia was 100 per cent negligent in failing to tell the hospital about a recent medication which Mr. Madonia testified he felt was unrelated to her presenting problems, when her next test according to her own cardiologist was not to occur for three more days.

[83] Counsel for Dr. Stevens' also submits that the administration of Propofol during the endoscopy on September 26 was a *novus actus interveniens*. This is

an affirmative defence and was not pleaded. It took the Plaintiffs by surprise.

Rule 25.07(4) of the *Rules of Civil Procedure* provides as follows:

In a defence, a party shall plead any matter on which the party intends to rely to defeat the claim of the opposite party and which, if not specifically pleaded, might take the opposite party by surprise or raise an issue that has not been raised in the opposite party's pleading.

[84] The rule is mandatory. If accepted, this defence would defeat the Plaintiffs' claim and it did take the opposite party by surprise. Counsel for Dr. Stevens tried to argue that since Dr. Wisenberg, the Plaintiffs' expert, raised the matter of Propofol it should not have surprised the Plaintiffs. This does not mean the Plaintiffs knew Dr. Stevens was relying on this as a defence. This is particularly so given what Dr. Wisenberg actually said about the Propofol, which will be detailed below. Counsel for Dr. Stevens could not have assumed from Dr. Wisenberg's testimony that the Plaintiffs were aware that Dr. Stevens was relying on Propofol as an affirmative defence (*novus actus interveniens*).

[85] Even if I permitted Dr. Stevens to raise this defence, Dr. Stevens has not established *novus actus interveniens* on a balance of probability and the onus is clearly on Dr. Stevens. Pages 415-417 of Allen M. Linden's text, *Canadian Tort Law*, 8ed (Butterworths) provides a useful overview of intervening medical error and the key points relevant to this case are as follows:

1. In *Mercer v. Gray*, [1941] OR 127 (C.A.) Justice McTague

indicated that if the treatment "is so negligent as to be actionable", it would be *novus actus interveniens*.

2. In *Papp v. Leclerc*, (1977), 77 D.L.R. (3d) 536 (Ont. C.A.)

Justice Lacoucière stated:

Every tortfeasor causing injury to a person placing him in the position of seeking medical or hospital help, must assume the inherent risks of complications, *bona fide* medical error or misadventure, and they are reasonable and not too remote ... It is for the defendant to prove that some new act rendering another person liable has broken the chain of causation.

3. Linden also makes reference to the decision in *Price v. Milawski* (1977), 82 D.L.R. (3d) 130 (O.C.A.) and quotes from Arnup J.A. as follows:

....a person doing a negligent act may, in circumstances lending themselves to that conclusion, be held liable for future damages arising in part from the subsequent negligent act of another, and in part from his own negligence, where such subsequent negligence and consequent damage were reasonably foreseeable as a possible result of his own negligence.

It was reasonably foreseeable by Dr. Murray that once the information generated by his negligent error got into the hospital records, other doctors subsequently treating the plaintiff might well rely on the accuracy of that information, i.e., that the X-ray showed no fracture of the ankle. It was also foreseeable that some doctor might do so without checking, even though to do so in the circumstances might itself be a negligent act. The history is always one factor in a subsequent diagnosis and the consequent treatment. Such a possibility was not a risk which a reasonable man (in the position of Dr. Murray) would brush aside as far-fetched -- see "Wagon Mound" (No. 2), *supra*, per Lord Reid at p. 643 G.

The later negligence of Dr. Carbin compounded the effects of the earlier negligence of Dr. Murray. It did not put a halt to the consequences of the first act and attract liability for all damage from that point forward. In my view the trial Judge was correct in holding that each of the appellants was liable to the plaintiff and that it was not possible to try to apportion the extent to which each

was responsible for the plaintiff's subsequent operation and his permanent disability.

[86] Through the evidence of Dr. Wisenberg, Dr. Stevens did not establish that the doctor administering Propofol was negligent and failed to meet the standard of care. As Dr. Wisenberg testified an endoscopic procedure was appropriate. The Propofol was used in the context of an appropriate medical investigation made necessary for Laura Madonia's bleeding.

[87] Dr. Wisenberg's evidence taken from his review of Laura Madonia's hospital records re: Propofol can be summarized as follows and it does not support an intervening negligent act:

1. Just at the beginning of her endoscopy on September 26, 2005 Laura Madonia was given a sedative Propofol. In addition to being a sedative, Propofol dilates blood vessels. In a patient such as Laura Madonia who has a hypertrophic cardiomyopathy, the Propofol would make her potential for obstruction, the blood leaving the left ventricle, significantly worse. The heart is trying to squeeze down to expel blood, but no blood is leaving the heart. Laura Madonia had a sudden increase in the degree of obstruction blood flow and that likely explains her cardiac arrest.

2. As a result, Laura Madonia's blood pressure fell markedly with inadequate perfusion to the brain. She was resuscitated, however due to the time her brain was without oxygen, "although the rest of her body had been resuscitated, her brain had been injured beyond the point of salvage".

3. Given the grave prognosis, the family decided to withdraw life support on October 2, 2005.

4. Dr. Wisenberg testified "It wasn't prudent" to administer Propofol to a patient who suffers from hypertrophic cardiomyopathy.

5. The decision to administer Propofol is made by the anaesthetist who is assisting with the endoscopic procedure.

[88] There was no other evidence before me with respect to use of Propofol. There was no evidence before me that the administering of Propofol fell below the standard of care for an anaesthetist and was a negligent act. The causation link was not broken by a *novus actus interveniens*.

9. Damage Assessment

[89] The award of damages for the Plaintiffs is restricted to what is provided in section 61 of the *Family Law Act*, R.S.O. 1990, c.F.3, as amended, namely the pecuniary loss of dependants plus a non-pecuniary loss related to "guidance,

care and companionship”.

[90] Section 61(1) of the *Family Law Act* provides as follows:

(1) If a person is injured or killed by the fault or neglect of another under circumstances where the person is entitled to recover damages, or would have been entitled if not killed, the spouse, as defined in Part III (support obligations), children, grandchildren, parents, grandparents, brothers and sisters of the person are entitled to recover their pecuniary loss resulting from the injury or death from the person from whom the person injured or killed is entitled to recover or would have been entitled if not killed, and to maintain an action for the purpose in a court of competent jurisdiction.

[91] Section 61(2) provides that:

- (2) The damages recoverable in a claim under subsection (1) may include,
- (a) actual expenses reasonably incurred for the benefit of the person injured or killed;
 - (b) actual funeral expenses reasonably incurred;
 - (c) a reasonable allowance for travel expenses actually incurred in visiting the person during his or her treatment or recovery;
 - (d) where, as a result of the injury, the claimant provides nursing, housekeeping or other services for the person, a reasonable allowance for loss of income or the value of the services; and
 - (e) an amount to compensate for the loss of guidance, care and companionship that the claimant might reasonably have expected to receive from the person if the injury or death had not occurred

[92] In assessing damages under the *Family Law Act* for loss of guidance, care and companionship, guidance can be found at para. 92 of *Kollaras (Litigation Guardian of) v. Olympic Airways S.A.*, [1999] O.J. No. 1447 (ON. S.C.) as to the relevant factors:

1. In order for a claim to succeed, there must be actual loss of care, companionship and guidance;

2. While the *Family Law Act* is remedial in nature, some measure of judicial restraint appears to be the order of the day;

3. No damages can be awarded for grief, sorrow or mental anguish by reason of an injury sustained by a relative;

4. Each claim must be assessed on its particular facts, although, a judge may have regard to:

(i) the age, mental and physical condition of the claimant;

(ii) whether the injured party lived with the claimant and, if not, the frequency of family visits;

(iii) the intimacy and quality of the claimant's relationship with the injured party;

(iv) whether or not the claimant is emotionally self sufficient; and

(v) the joint life expectancy of the claimant and the injured party (not relevant to the matters in issue).

[93] The uncontroverted evidence before the Court is that Laura Madonia's life expectancy prior to the negligent acts of Dr. Stevens was 2 years. As a result, in assessing an award for loss of care, guidance and companionship, the Court must consider among all the contingencies, including the intimacy and quality of

the relationship, the short life expectancy of Laura Madonia and the probable length of time the relationship might be expected to continue.

Salvatore Madonia

Is Salvatore Madonia entitled to non-pecuniary general damages in relation to the death of his wife, Laura Madonia?

Law

[94] Section 61(2) of the *Family Law Act* states that Mr. Madonia, as spouse of Laura Madonia, shall be entitled to an amount to compensate for the loss of guidance, care and companionship that he might reasonably have expected to receive from his wife, Laura Madonia, but for her death on October 2, 2005.

[95] In the case of *Riggs v. Toronto General Hospital*, [1993] O.J. No. 1884 (Gen'l Div), the deceased, Mrs. Riggs, was 71 at her death. Her death was the result of medical negligence at the Toronto General Hospital. The deceased had a life expectancy of approximately 15 years. The surviving spouse, Mr. Riggs, was 77 at the time of his wife's death. They were constant companions and inseparable. The non-pecuniary general damages were assessed at \$50,000.

[96] In the case of *Hechavarria v. Reale*, [2000] O.J. No. 4288 (ON. S.C.), the Court awarded a general damage assessment to the surviving spouse of

\$85,000. In this case, Mr. Hechavarria's wife (age 53) was killed when her vehicle was struck by a school bus.

[97] The case of *Fish v. Shainhouse*, [2005] O.J. No. 4575 (ON S.C.) is a medical negligence case. Liability was not proved but the spouse of Mr. Fish was assessed \$80,000 in non-pecuniary general damages. The deceased was 54 years old and the Court accepted that his 5 year survival rate was 50% for a 5 year period.

[98] In the case of *Stephen v. Stawecki*, [2006] W.D.F.L. 2934, the Ontario Court of Appeal awarded \$70,000 to a "spouse" for the loss of guidance, care and companionship in circumstances where the spouse and the deceased maintained separate residences. There is no discussion with respect to the life expectancy of the deceased or the surviving spouse.

[99] In the case of *Rupert v. Toth*, [2006] 38 C.C.L.T. (3d) 261 (ON S.C.), the Court awarded the surviving spouse \$70,000 for the loss of guidance, care and companionship. The evidence was that Loraine and Pat Rupert were childhood sweethearts. They had separated for a long period of time due to Pat's alcoholism. They had gotten back together in "middle age" and fell in love again. They ran a daycare business together. This was a medical negligence claim. The trial judge had concluded that the deceased's life expectancy was 5 to 7 years.

[100] With respect to the case of *Wilcox v. "Miss Megan" (The)* 2007 CarswellNat. 3324 (Fed'l Ct) the deceased was 63 years old and the Court accepted that the actuary correctly assumed the deceased would have lived to the age of 75. The surviving spouse was awarded \$75,000 for the loss of guidance, care and companionship. The Court was applying an Act similar to the *Family Law Act*.

[101] Mrs. Madonia's life expectancy was less than in any of the above-referenced cases.

Evidence

[102] Salvatore and Laura Madonia were married on June 11, 1950. Laura Madonia worked as a sales clerk until about 1957 and did not work outside the home from that point forward. Laura Madonia was primarily responsible for raising the children, especially when Salvatore Madonia traveled with his job. They were lifelong companions and would do many things together. This was even more so after Salvatore Madonia retired from work. He did not do much without her. Mr. Madonia dearly misses his wife and her companionship and the things that they used to do together. He misses her smile, her company, and her love and care. His relationship with his wife meant the world to him. Mr. Madonia describes her as a great woman, good wife, and good provider for the family. They would discuss things together quite a bit before they did anything.

Salvatore Madonia no longer has this companionship. He no longer has Laura Madonia to take care of him. Laura Madonia would do all of the cooking, the majority of the cleaning and the gardening. He now eats most of his meals out or eats pre-packaged meals. He is very lonely and withdrawn without her.

[103] Mr. Madonia is currently 80 years old and continues to live alone in the matrimonial home. Other than cooking, he is managing the necessities of life competently and self-sufficiently.

Finding

[104] The Plaintiffs have suggested that an appropriate award is \$75,000 and the Defendant suggests that the award should not exceed \$30,000. I find that an appropriate non-pecuniary general damage assessment for Salvatore Madonia is \$50,000. I have considered the effect of inflation on the awards referred to in the above referenced cases.

Is Salvatore Madonia entitled to pecuniary damages?

[105] There are three aspects to this damage claim:

1. the direct loss attributable to the reduction in Salvatore Madonia's pension resulting from the death of this spouse;

2. the loss of the portion of Laura Madonia's pension to which Salvatore benefited from; and
3. the value of the domestic services that Laura Madonia provided prior to her death.

Law

[106] Section 61 of the *Family Law Act* provides that Salvatore Madonia is entitled to recover his pecuniary losses resulting from the death of his wife. This would include any loss of income sustained by him and his share of Laura Madonia's income that he benefited from prior to her death.

[107] In the case of *Riggs v. Toronto Hospital (supra)*; *Hechavarria v. Reale (supra)*; *Rupert v. Toth (supra)* and *Wilcox v. "Miss Megan" (supra)*, the Courts awarded the surviving spouse dependency losses in relation to the deceased spouse's pre-death income. These cases also awarded damages in relation to the value of the loss of domestic services that the surviving spouse has lost as a result of the death of a spouse who provided these services prior to death.

Evidence

1. *Salvatore Madonia's Pension*

[108] Salvatore Madonia testified that he received a series of pensions relating to his employment in the United States and in Canada. He also has a Canadian RRSP and a U.S. Social Security Pension. He testified that his annual pension income is \$117,000.

[109] Mr. Madonia's pension states that if either Salvatore or Laura passed away, the pension would be reduced by one third. In the Plaintiffs' Economic Loss Brief (Exhibit 4) is a letter from McDermott Retirement Service Center dated January 26, 2006 which states as follows:

Under the terms of the Retirement Plan and in accordance with the 66 2/3% Joint and "Last" Survivor Annuity option you elected for January 1, 1993, your revised benefit at November 1, 2005 (the first month following the death of your spouse, Laura Madonia) should have been \$1,223.20 per month. This option provides that upon the death of your former spouse, you would receive 66 2/3% of the original monthly payment for life.

We have applied the terms of your option to your February 1, 2006 payment, reducing the payment from \$1,834.80 to \$1,223.20 per month for life.

[110] Thus, there is a direct monthly loss to Salvatore Madonia of \$611.60 per month.

[111] Brent Pyper was called as an expert witness by the Plaintiffs and was qualified to give opinion evidence with respect to quantifying the economic losses in this case. He assumed a 2 year life expectancy for Laura Madonia but for her death on October 2, 2005. As such, he calculated the loss relating to Salvatore Madonia's reduced pension amounts to \$15,941.

[112] The Defendant argues that a “dependency factor” type calculation should be applied to this aspect of the loss in a similar manner as Mr. Pyper testified should be applied to the loss of Laura Madonia’s pension detailed below. The Defendant argues that some deduction needs to be made to account for the assumption that some portion of this amount, if not all of it, will no longer be required or used to support Laura Madonia.

[113] Alternatively, the Defendant argues that if no deduction is made to reflect the “savings” associated with Laura Madonia’s cost of consumption, a cross-dependency approach should be applied. Mr. Pyper described this approach but did not apply it to this situation. Under this approach, you add the incomes of the spouses together and assume that one person typically requires about 70% of the income of a family of 2, to maintain the same standard for a family of one. 70% is applied to the combined earnings to determine the necessary amount and from that the survivor’s income is deducted. This would result in an economic loss of zero in this case.

[114] I reject both of the Defendant’s arguments for the following reasons:

1. This is a direct financial loss to Mr. Madonia. It is not the same as Mrs. Madonia’s loss of income which Mr. Madonia would have benefited from in part. The calculation of loss arises from a contractual agreement. This is income he otherwise would have received directly except for the

death of Laura Madonia. It is not a function of the dependency he had on his wife's income.

2. The Defendant did not provide any case law authority where such deductions/calculations were done to a direct financial loss suffered by the survivor as a result of the death of his or her partner. The authority referenced below was a loss of income of the deceased. There is no judicial authority for applying a dependency factor to a direct loss of a surviving spouse.

3. It is not inconsistent to use different approaches as one is a direct loss to the survivor and the other is a loss of the deceased's income, a portion of which the survivor would have benefited from.

4. The Defendant did not call any expert to testify as to economic loss calculations. Mr. Pyper answered the Defendant's questions in cross-examination, but never waived from his view as to the appropriate calculations in this case.

2. *Dependency Claim = Laura Madonia's Pension*

[115] Salvatore Madonia testified that Laura Madonia's income was in the \$12,000 range. This income was derived from a U.S. social security pension.

[116] The income tax returns of Laura Madonia were filed in the Plaintiffs' Economic Loss Brief (Exhibit 4). In 2004, Laura Madonia's total income was \$12,418.47.

[117] Mr. Pyper advised the Court that he assumed a dependency factor of 60%. Mr. Pyper used the "modified sole dependency theory". In a single income family, the dependency factor is 70%. In a dual income family, it is modified to 60%. In other words, 60% of Laura Madonia's income would go to the family and 40% would be spent on herself.

[118] The Court of Appeal in Ontario considered similar facts in the case of *Nielsen v. Kaufman* (1986), 54 O.R. (2d) 188 and stated as follows:

Accepting the trial Judge's view that the deceased was an intelligent and thrifty wife and mother, who was prepared to sacrifice her interests to those of her husband and children, some offset must still be allowed for the fact that no portion of Mr. Nielsen's income will now be going to support his wife. This was a family where there was obviously a pooling of the resources and where the death of one partner would have an impact with some offsetting "credit". Taking the recited factors into account, we would allot 60 per cent to Mr. Nielsen and 4 per cent to each child.

[119] In the Madonia family, the incomes of both Mr. and Mrs. Madonia were pooled and used for family expenditures. All of the income was placed in one common bank account. This suggests that the 60% dependency factor used by Brent Pyper and used in *Nielsen v. Kaufman* is appropriate in this case.

[120] Mr. Pyper calculated the dependency loss at \$14,261.00. The Defendant did not oppose Mr. Pyper's calculation of the dependency loss of Laura Madonia's pension, but wanted a similar calculation done with respect to Mr. Madonia's pension as set out above.

3. *Loss of Domestic Services*

[121] Salvatore Madonia testified that Laura was the head cook and did most of the house cleaning. She also did a lot of the gardening. He would help with the vacuuming, wash pots and pans and help make the bed. He testified that although he tried to help, she was in charge. They would shop together but he would essentially just follow her around. They were comfortable in their home and had no intentions of moving out into an assisted living environment. As of September, 2005, Laura was still doing the cleaning, cooking and gardening.

[122] Brent Pyper assumed a figure of \$30 per week for 50 weeks of the year as the cost of replacing the domestic services provided by Laura Madonia. This accounts to \$1,500 per year and \$3,000 for 2 years. Brent Pyper suggests that this is a very reasonable figure, given that the market rate for a homemaker is \$21.29 per hour as stated in the Rehabilitation Management Inc. letter dated February 26, 2008 filed in the Plaintiffs' Economic Loss Brief. The \$30 per week is the equivalent to about 1.5 hours of a homemaker at current market rates. The Defendant did not dispute this calculation.

Finding:

[123] I make the following findings re: pecuniary damages:

Salvatore Madonia's Pension Loss:	\$15,941
Dependency Claim Re: Laura Madonia's Pension	\$14,261
Loss of Domestic Services	<u>\$ 3,000</u>
TOTAL	<u>\$33,202</u>

Is Salvatore Madonia entitled to special damages in relation to funeral expenses?

Law:

[124] Section 61(2)(b) of the *Family Law Act* is clear that in a wrongful death claim, the spouse can recover "actual funeral expenses reasonably incurred."

Evidence:

[125] The Plaintiffs' Special Damage Brief was made Exhibit 3. The expenses claimed are as follows:

Funeral Expenses (Oakville Funeral Home)	\$10,428.13
Burial Plot (Memorial Gardens Canada Limited)	
(\$11,058.45 divided by 2)	\$ 5,529.23
Funeral Dinner for out-of-town guests	<u>\$ 1,693.66</u>

TOTAL

\$17,651.02

[126] The Defendant does not object to these claims except for the dinner for out-of-town guests. The Defendant takes the position that the dinner expense is not properly recoverable. I disagree. It was entirely reasonable and part of the overall funeral costs for Mr. Madonia to provide a reception for guests that had travelled some distance to attend the funeral.

Finding:

[127] These are all reasonable funeral expenses as contemplated by s. 61(2)(b) of the *Family Law Act* and I find that damages in the amount of \$17,651.02 shall be awarded.

Paul Thomas Madonia (son)

Is Paul Madonia entitled to non-pecuniary general damages in relation to the death of his mother, Laura Madonia?

Law:

[128] Section 61 of the *Family Law Act* provides that a child is entitled to advance a claim for the loss of guidance, care and companionship as a result of

the wrongful death of his mother. The following cases have damage assessments for children:

<i>Riggs v. Toronto Hospital (supra)</i>		\$10,000
<i>Adair Estate v. Hamilton Health Sciences Corp,</i> <i>2005 CarswellOnt 2180 ON S.C.)</i>		\$20,000
<i>Hechavaria v. Reale (supra)</i>		\$30,000
<i>Wilcox v. "Miss Megan" (supra)</i>		\$25,000
<i>Simpson Estate v. Cox</i> <i>2006 Carswell NS 135 (S.C.)</i>		\$10,000
<i>Nielsen v. Kaufman (supra)</i>	one child	\$30,000
	one child	\$20,000

Evidence

[129] Paul Madonia was adopted by Salvatore and Laura when they were living in New York in or around 1956. From 1972 to 1984 the family lived in Oakville. In 1984 his parents moved to Ohio and Paul Madonia stayed in Canada. He would see his parents 5 to 6 times per year as it was only a 4 to 5 hour drive and he would go down for weekends. When his parents were living in Atlanta from 1986 to 2000, he would see them 1 to 2 times per year. He would stay in touch with his mother by telephone and they would speak every other day. His parents would also come from Atlanta to visit him and his family. When his parents moved back to Burlington in 2000, he would see his mother every

other weekend. They would also get together for family dinners, sometimes at his house and sometimes at his parents.

[130] Paul Madonia testified that his mother has always been the “glue” in the family. She made sure that they all kept in touch and she would remind him of things such as sending a birthday card to his sister. When Paul Madonia was going through his divorce, he describes his mom as his “best friend”. After the divorce he would talk to her about a lot of things and they were very close. During the last two years of Laura Madonia’s life, he would talk to her every other day on the telephone. Paul Madonia is the father is Katheryn and Alexander Madonia.

Finding:

[131] The Plaintiffs have requested an award of \$30,000. The Defendant’s position is that the award should not exceed \$10,000. An appropriate award for the loss of guidance, care and companionship that Paul Madonia has suffered as a result of the loss of his mother is \$20,000. This is reasonable in light of the closeness of the relationship and given that comparable awards must be indexed for inflation.

Laura Theresa Lewis (daughter)

Is Laura Lewis entitled to non-pecuniary general damages in relation to the death of her mother, Laura Madonia?

Law

[132] Section 61 of the *Family Law Act* provides that a child is entitled to advance a claim for the loss of guidance, care and companionship as a result of the wrongful death of her mother. The following cases have comparable damage assessments for children:

<i>Riggs v. Toronto Hospital (supra)</i>		\$10,000
<i>Adair Estate v. Hamilton Health Sciences Corp, (supra)</i>		\$20,000
<i>Hechavaria v. Reale (supra)</i>		\$30,000
<i>Wilcox v. "Miss Megan" (supra)</i>		\$25,000
<i>Simpson Estate v. Cox (supra)</i>		\$10,000
<i>Nielsen v. Kaufman (supra)</i>	one child	\$30,000
	one child	\$20,000

Evidence

[133] Laura Lewis is the adopted daughter of Salvatore and Laura Madonia. She was born on April 1, 1961. She is the mother of Walker Lewis.

[134] When her parents moved to Ohio in 1984, she stayed in Canada. She would drive to visit her parents almost every weekend. This trip would take 4 hours and 20 minutes. She would go out to dinner with her parents and help her mom with the gardening and shopping. Her parents would also come to visit when they lived in Ohio for holidays and birthdays.

[135] In 1986, her parents moved to Atlanta. She would go and visit them 4 to 5 times per year. All of her vacation time was spent with her parents. She would continue to spend time helping her mother and would also spend a lot of time around the pool. Her parents would also come up to Canada 3 to 4 times per year to visit her and her family when her parents were living in Atlanta.

[136] When her parents moved to Burlington in 2000, she saw them very often. She had cut back her hours at work to allow her to spend more time with her parents. She would see them 4 to 5 times per week. She would go for dinner at her parents' house with her son Walker, go out for lunch with her mom, go shopping with her mom and would help her mom with her garden. When her mother had surgery in Florida, she spent a month with her mother helping her to recover.

[137] Laura Lewis was diagnosed with breast cancer in 2003. This was a very sad and terrifying time for her. Her parents would go to her oncology appointments with her and would also come with her to her treatments. Her

mother cooked most of the meals for her family during this time period. She was on chemotherapy from September 2003 to January 2004. Her mother was by her side the whole time and would rub her back and hold her hand. Her mother was a source of great comfort and strength during this time.

[138] Laura Lewis describes her mother as her best friend; the kindest, most loving and generous, caring mother in the world. They talked about everything and her mother would constantly give her advice.

Finding:

[139] The Plaintiffs request an award of \$30,000. The Defendant suggests that the award should not exceed \$10,000.

[140] An appropriate award for the loss of guidance, care and companionship that Laura Lewis has suffered as a result of the loss of her mother is \$20,000. This is reasonable in light of the closeness of the relationship and given that comparable awards must be indexed for inflation.

Katheryn Anne Madonia, Alexander Gregory Madonia and Walker Michael Salvatore Lewis (grandchildren)

Are the grandchildren entitled to non-pecuniary general damages in relation to the death of their grandmother, Laura Madonia?

Law:

[141] Section 61 of the *Family Law Act* provides that a grandchild is entitled to advance a claim for the loss of guidance, care and companionship as a result of the wrongful death of his or her grandmother. The following cases have damage assessments for grandchildren:

<i>Riggs v. Toronto Hospital (supra)</i>	\$ 5,000
<i>Adair Estate v. Hamilton Health Sciences Corp, (supra)</i>	\$5,000-10,000
<i>Fish v. Shainhouse (supra)</i>	\$20,000
<i>Simpson Estate v. Cox (supra)</i>	\$10,000

Evidence

1. Katheryn and Alexander Madonia

[142] Katheryn was born March 23, 1990 and Alexander was born on January 12, 1993. From 1986 to 2000, Salvatore and Laura Madonia lived in Atlanta. Paul Madonia testified that he would visit his parents in Atlanta with his children 1 to 2 times per year. Salvatore Madonia testified that they would come up to visit their children and grandchildren 3 to 4 times per year when they lived in Atlanta. They would also call the kids quite often and actually bought telephone call cards so that the children and grandchildren could call without them having to spend money on long distance calls.

[143] In 2000, Salvatore and Laura Madonia moved to Burlington to be closer to their children and grandchildren. Paul Madonia testified that his children would see their grandmother every other weekend. Laura Madonia would be involved at school events and events such as Brownies. In addition to the visits every other weekend, Katheryn and Alexander would see Laura Madonia an additional 3 to 4 times per month during the months that the children were in Paul Madonia's care (he and the children's mother were divorced in 2001 and he and his former wife alternated care of the children). Paul Madonia acknowledged that the children saw Laura Madonia less often when they were with their mom, but that they would also keep in touch by telephone. The children would also help their grandmother with weeding the garden.

2. Walker Michael Salvatore Lewis (grandson)

[144] Walker had a particularly close relationship with his grandmother. He would sleep over at this grandparents' house at least twice a month and sometimes every weekend. In the summer, he loved to swim in their pool and would sometimes bring his friends.

[145] Laura Lewis, Walker's mother, testified that he called his grandmother "nanny". She was the most important person in the world to him. He spent a great deal of time with her playing checkers, chess, cards, watching movies, going out for dinner and helping her cook. Walker and his grandmother

would also plant a vegetable garden together every year and looked after it together. He would love to listen to his grandmother talk about stories from her childhood, often asking her to tell him the stories over and over again. She would go to his baseball games and never missed a school event. When they went on vacation together, they would go for long walks together collecting shells.

Finding:

[146] The Plaintiffs suggest an award of \$10,000 each for Katheryn and Alexander and \$15,000 for Walker. The Defendant's position is that the award for Katheryn and Alexander should not exceed \$2,500 each and Walker's should not exceed \$5,000.

[147] Appropriate awards for the loss of guidance, care and companionship that the grandchildren have suffered as a result of the loss of their grandmother are as follows:

Katheryn Anne Madonia	\$7,500
Alexander Gregory Madonia	\$7,500
Walker Michael Salvatore Lewis	\$12,500

OHIP's Subrogated Claim

[148] Section 30(1) of the *Health Insurance Act* R.S.O. 1990 c.H. 6, states as follows:

Subrogation

30.(1) Where, as the result of the negligence of other wrongful act or omission of another, an insured person suffers personal injuries for which he or she receives insured services under this Act, the Plan is subrogated to any right of the insured person to recover the cost incurred for past insured services and the cost that will probably be incurred for future insured services, and the General Manager may bring action in the name of the Plain or in the name of that person for the recovery of such costs.

[149] The Plaintiffs' Special Damages Brief (Exhibit 3) contains the details of OHIP's Subrogated Claim. The Payment Summary lists the services provided to Laura Madonia from September 26, 2005 until the date of her death on October 2, 2005. The amount of OHIP's claim is \$6,607.09. The Defendant did not dispute this amount and I find that the Defendant shall be liable for same.

Prejudgment Interest

Non-Pecuniary General Damages

[150] Pursuant to s.128(2) of the *Courts of Justice Act*, pre-judgment interest should be awarded on all non-pecuniary general damage assessments at the rate determined by the Rules of Court. Rule 53.10 sets the rate at 5% per annum. Pre-judgment interest shall be paid from the date of death (October 2, 2005) to the date of judgment at this rate.

Special Damages

[151] Section 128(3) of the *Courts of Justice Act* states that the interest calculated on pecuniary losses shall be calculated on the total past pecuniary loss at the end of each six month period and at the date of the Order. The chart of pre-judgment interest rates per respective quarter is found at s. 127 of the *Courts of Justice Act*. The average quarterly rate over the two-year period commencing from October 2, 2005 is 4.05%.

1. Salvatore Madonia's Pension Reduction

[152] The Plaintiffs suggest that the Court use a "half rate" of 2.025% to account for the fact that Mr. Madonia would have received the money at different times over the two years proceeding Laura Madonia's death. The Defendant did not dispute this approach and therefore I order same. From October 2, 2007 to the date of Judgment, interest should be paid at the rate of 4.5%

2. Dependency Claim

[153] The Plaintiffs suggest that the Court use a "half rate" of 2.025% to account for the fact that Mr. Madonia would have received the money at different times over the two years proceeding Laura Madonia's death. The Defendant did not dispute this approach and therefore I order same. From October 2, 2007 to the date of Judgment, interest shall be paid at the rate of 4.5%.

3. Loss of Domestic Services

[154] The Plaintiffs suggest that the Court use a “half rate” of 2.025% to account for the fact that Mr. Madonia would have received the services at different times over the two years proceeding Laura Madonia’s death. The Defendant did not dispute this approach and therefore I order same. From October 2, 2007 to the date of Judgment, interest shall be paid at the rate of 4.5%.

4. Funeral Expenses

[155] The funeral expenses were all incurred by Mr. Madonia on or about October 7, 2005. The rate applicable, as per s. 128(3) of the *Courts of Justice Act*, would be at the rate each 6 month period. The average rate over this time period of 4.05%. The award of damages under this heading shall bear interest at the rate of 4.05% per annum from October 7, 2005 to the date of Judgment.

5. OHIP

[156] The medical expenses paid out to the various members of the medical profession who provided services to Laura Madonia while she was in hospital were incurred from September 26, 2005 to October 2, 2005. The rate applicable, as per s. 128(3) of the *Courts of Justice Act*, would be the rate of each 6 month period. The average rate over this time period if 4.05%. The

award of damages under this heading shall bear interest at the rate of 4.05% per annum from October 2, 2005 to the date of Judgment.

9. Summary

[157] The Plaintiffs shall have judgment in accordance with the Reasons for Judgment set out above.

[158] With respect to costs, if counsel are unable to resolve the issue of costs, they may schedule a time before me through the trial coordinator for oral submissions. Written submissions and Bill of Costs shall be exchanged and filed in advance of the oral submissions.

Coats J.

Released: December 12, 2008

COURT FILE NO.: **2895/06**
DATE: **20081212**

ONTARIO

SUPERIOR COURT OF JUSTICE

B E T W E E N:

SALVATORE MADONIA, PAUL
THOMAS MADONIA, KATHERYN
ANNE MADONIA, a minor under the
age of eighteen years by her litigation
Guardian, Paul Thomas Madonia,
ALEXANDER GREGORY MADONIA, a
minor under the age of eighteen years
by his litigation Guardian, Paul Thomas
Madonia, LAURA THERESA LEWIS,
Personally and WALKER MICHAEL
SALVATORE LEWIS, a minor under the
age of eighteen years by his litigation
Guardian, Laura Theresa Lewis

Plaintiffs

- and -

Brett J.D. Stevens

Defendant

REASONS FOR JUDGMENT

Coats J.

Released: **December 12, 2008**